breast cancer now

We are family

Annual Report and Accounts 2015/16

We are family

We're determined to stop women dying from breast cancer. We know it can be done.

But it's only by standing together – as scientists, policy makers, fundraisers, families, colleagues and campaigners – that we will make it happen.

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One year closer

Following the launch of Breast Cancer Now in June 2015, we have been delighted by the public's response. We believe that if we all act now, by 2050, everyone who develops breast cancer will live.

One year on from the merger of Breast Cancer Campaign and Breakthrough Breast Cancer we are demonstrating that by uniting to stop breast cancer taking lives we are already having a truly positive impact across the UK for people affected by the disease.

Despite a challenging time for the sector we saw our total income rise to £27.6 million and made considerable savings in our expenditure. Most importantly, we have also been able to increase our investment in our charitable work across the UK by nearly £2 million.

High-quality research is the key to stopping breast cancer taking lives. Our scientific research community is focused on addressing the critical gaps in breast cancer knowledge more quickly. We're supporting nearly 400 researchers, including many of the world's top breast cancer scientists, at more than 30 institutions across the UK and Ireland. This includes The Breast Cancer Now Toby Robins Research Centre – the UK's first dedicated breast cancer research centre.

This year 145 scientific publications have been published, including a study by Professor Clare Isacke which identified a key protein that enables cancer cells to spread and eventually become incurable. This finding brings us a step closer to understanding how secondary breast cancer develops and may help us identify those patients most at risk.

Collaboration is at the heart of our charity, and creating new partnerships and relationships has been a major focus of our work. In February 2016 we announced the launch of the Breast Cancer Now Catalyst Programme. By inviting the world's leading researchers and pharmaceutical companies to pool resources, this new initiative will be a great boost for breast cancer research in Europe.

Through our first collaboration with Pfizer, one of the world's largest research-based pharmaceutical companies, our researchers will be granted unprecedented access to at least 13 of Pfizer's existing drugs and drugs currently in development. Pfizer has also committed around £10 million in funding over three years to ensure research can happen.

Whilst high-quality research is the key to stopping breast cancer taking lives, research alone isn't enough. That's why we use our influence to drive improvements in policy and practice and create the change needed so patients across the UK can benefit from scientific progress. Through our campaigning work we endeavour to ensure that everyone affected by the disease receives the best possible treatment and services.

Earlier this year, we were invited to sit on the National Cancer Advisory Group, as a representative of the Common Cancers Coalition, to help advise on the implementation of the Cancer Strategy. We continued to help patients get access to the drugs they need in England and Wales. More than 42,000 people joined our call urging Roche to reduce the price of the innovative secondary breast cancer drug Kadcyla, so it could remain available in England through the Cancer Drugs Fund. It's thanks to the strength and passion of our supporters that our voices were heard and women with secondary

breast cancer are still able to benefit from this drug. However, larger problems remain with our drug access and pricing systems, and we will continue to work with patients, pharmaceutical companies and the NHS across the whole of the UK to ensure patients can access the treatments they need.

We have continued to raise the profile of breast cancer and our work to stop the disease, among politicians across the UK. In March 2016, every mainstream political party in Scotland got behind Breast Cancer Now's 2050 Challenge. As a result, the Scottish Government has made a clear commitment within its Cancer Strategy to adopt our 2050 ambition.

During a challenging year for the charity sector, we continued to work to ensure we use our supporters' generous donations as effectively as possible. Our aim is to inspire people to support Breast Cancer Now and ensure they feel valued and respected by the charity and can see the life-saving impact of their donations. Breast Cancer Now will be joining the new Fundraising Regulator and currently works to ensure our fundraising reflects not only regulatory requirements but best practice. Thanks to the fantastic support and commitment of thousands of individuals, companies and trusts, our income has risen.

In the coming year, we will also work with the scientific community to analyse the effects of Brexit on our work, help mitigate the risks and maximise any opportunities it may bring. We will continue to seek post-merger efficiencies and find savings in our day-to-day work, to ensure we maximise our impact on breast cancer at every opportunity.

As we look to the future, we're developing a new strategy for Breast Cancer Now and identifying the steps we need to take to stop this dreadful disease. The strategy is informed by leading scientists and experts, and includes direct input from thousands of people diagnosed and living with breast cancer including through our new initiative, The Big Breast Cancer Conversation. We are committed to hearing the views of all people affected by this disease and will use their feedback to guide our direction and assess where we can achieve the greatest impact.

We firmly believe that if we all act now, by 2050, everyone who develops breast cancer will live. But we can only make this happen if we harness the passion and ambition of our supporters – they are our heart, and together we will stop breast cancer and save lives.

Baroness Delyth Morgan
Chief Executive

Professor Lynne Berry
Chair of Trustees

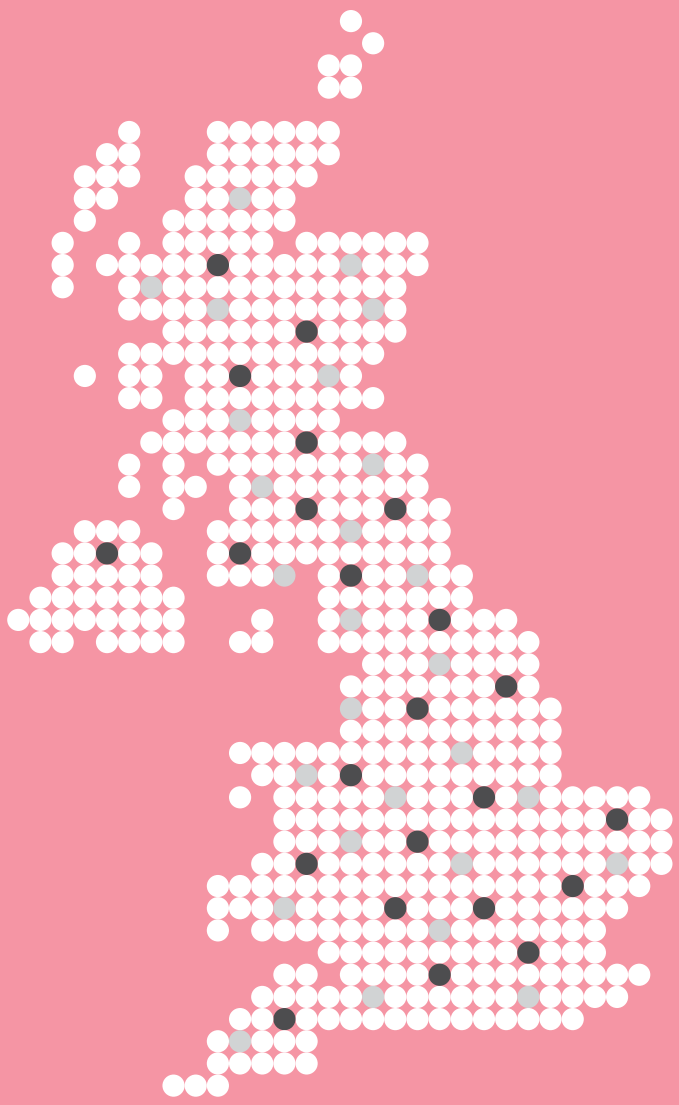
The Trustees are aware of the Charity Commission's guidance on public benefit and confirm that they have complied with the duty in section 17 of the Charities Act 2011.

About Breast Cancer Now

We're the UK's largest breast cancer charity – and we're dedicated to funding research into this devastating disease.

We believe that if we all act now, by 2050, no one who develops breast cancer will die from it. But we'll only reach that point by working in a new, collaborative way. We're bringing together all those affected by the disease to improve the way we prevent, detect, treat and stop breast cancer.

Breast Cancer Now won't stop until breast cancer has taken its last life; something we are confident we can achieve, but only if we get the support we need.



Right now, we are funding over £26 million worth of ground-breaking research projects, supporting nearly 400 of the world's brightest researchers at more than 30 institutions across the UK and Ireland.

About breast cancer

Breast cancer is the most common cancer in the UK. One in eight women will face it in their lifetime.

Nearly 700,000 people living in the UK have experienced a diagnosis. More women are being diagnosed with breast cancer than ever before.

This year, more than 50,000 women will be told they have the disease. That's one woman every 10 minutes.

Breast cancer touches the lives of millions of us in the UK. For every woman or man who develops breast cancer, there are families, friends and loved ones who also feel the fear and uncertainty the disease brings.

Nearly 1,000 women in the UK die from breast cancer every month. That's one woman every 45 minutes whose life is cut short by the disease.

It's been a busy year... Strategic report 2015-16

In 2015 we published our Action Plan for Breast Cancer which sets out the key challenges we must overcome if we're to achieve our ambition that by 2050, everyone who develops breast cancer will live.

Throughout this publication we are reporting against the aims and objectives of Breast Cancer Now from 1 August 2015 to 31 July 2016.

The following summarises this year's achievements and performance as measured against these areas of focus:

Risk and Prevention

Early detection and diagnosis

Treatment

Secondary breast cancer

Our work also promotes collaboration and innovation. We connect the laboratory bench with the hospital bedside, the GP's surgery, the MP's office and the policies that govern our health service – and all of our work is informed by women and men living with breast cancer and those committed to putting an end to the disease.

Risk and prevention

Our aims and results in 2015-2016

We will understand how genetic, hormone and lifestyle factors lead to the development of breast cancer, and use this knowledge to accurately assess an individual's personal risk of developing the disease.

The Breast Cancer Now Generations Study, which has now been running for 12 years, continued to make great progress in understanding the causes of breast cancer. In its latest analysis, the study found that day-to-day psychological stress and adverse life events are unlikely to increase a woman's risk of developing breast cancer. Since its launch, the Breast Cancer Now Generations Study has contributed to the discovery of over 90 genetic variants associated with breast cancer risk – a number set to rise again in the coming years.

This may ultimately allow women to have their DNA tested to build a more accurate picture of their individual risk of breast cancer. We believe this could lead to the development of a test to predict whether a woman has a higher risk of developing the disease.

We will identify sustainable lifestyle changes, such as diet and physical activity, which can reduce the risk of breast cancer, and help women understand how making these changes could benefit them.

Over the past year, we have reached more than 200,000 people with our information on breast cancer risk. This includes printed publications, website views and downloads and social media engagement. We also responded to the new guidelines on alcohol issued by the Chief Medical Officer, providing updated information on our website and a blog which we shared on social media. We also published a blog on breast cancer myths, which was viewed almost 5,000 times on our website and has reached over 40,000 people on social media.

The past year has also seen the development of the Marks & Spencer wellbeing programme, which aims to provide information and behaviour change tools to encourage M&S staff and customers to make lifestyle changes in order to reduce their risk of breast cancer. This five year project is part of our partnership with M&S that focuses on prevention.

"Our study has analysed very large amounts of data from women over many years, and has provided good evidence that stress is unlikely to increase the risk of developing breast cancer."

Dr Minouk Schoemaker
The Institute of Cancer Research, London.

We will find the best risk-reducing interventions for women at increased risk of breast cancer and ensure these are made available and used as effectively as possible.

We have continued to fund several studies that are investigating how to predict which women will benefit from chemopreventative drugs, as well as research looking at whether a drug called ulipristal acetate can be used to prevent breast cancer in women classified as at high risk.

We will ensure that the policy and public health environment responds to the latest evidence to deliver effective risk and prevention tests and treatments, as well as policies to support healthy, active lifestyles and improved public health in order to prevent more breast cancer cases.

In March 2016, the Scottish Government committed to exploring the potential of ActWELL, a breast cancer risk reduction programme offered to women attending routine breast screening. We campaigned for the introduction of ActWELL as part of the 2050 Challenge campaign and will now be a partner in the delivery of a full-scale trial of the programme.

Early detection and diagnosis

Our aims and results in 2015-2016

We will find new ways to detect breast cancer at its earliest stages and ensure that this knowledge is implemented.

Detecting breast cancer early is essential if we are to give women the best possible chance of survival. That's why we are funding projects to develop new, non-invasive imaging techniques that will spot the signs of breast cancer earlier and help to identify those most at risk of developing the disease.

We will monitor developments in the stage at which breast cancer is detected across different areas and groups and raise awareness of the signs and symptoms of breast cancer, giving women the confidence to take any concerns to their doctor as quickly as possible.

As part of our ambition to encourage women to be breast aware, we have developed a new smartphone app, Breast Check Now, which will be launched in Autumn 2016. Breast Check Now aims to help women form a habit of checking their breasts regularly. It will replace our original app, iBreastCheck, which - following a rebrand in early 2016 - has been downloaded around 100 times per month.

Our information on early diagnosis, particularly our Touch Look Check (TLC) campaign, has reached over 1.4 million people through our printed publications, website visits and social media. Our online guide to breast screening has also been viewed 7,600 times.

We will develop better methods of differentiating between very early breast cancers that require treatment and those that don't, avoiding over-diagnosis and over-treatment.

Our scientists have been analysing the genetic make-up of a non-invasive cancer known as ductal carcinoma in situ (DCIS) to find out how to predict whether a woman with DCIS will develop invasive breast cancer. Around half of DCIS cases will become invasive if left untreated, so it is important that we can accurately identify which patients will develop invasive breast cancer and which can be spared unnecessary treatment.

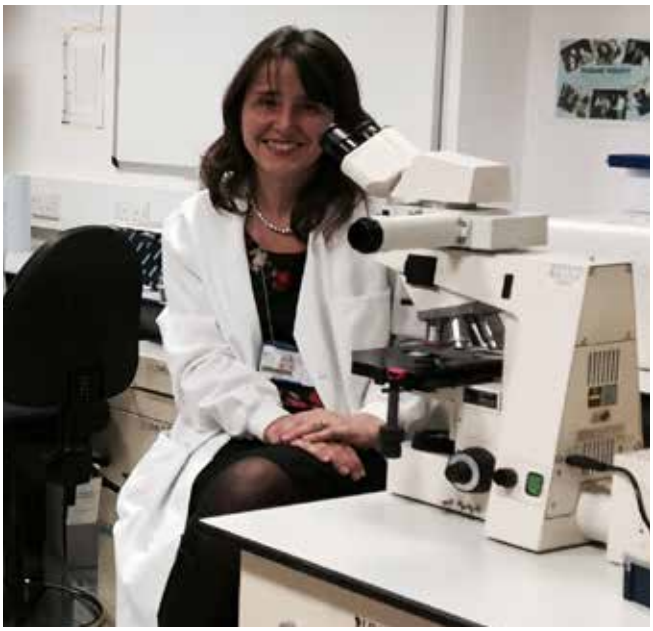
We will work with government, clinicians and the NHS to improve earlier diagnosis and detection by monitoring and influencing the quality of breast cancer services, including GP referral pathways, to ensure they're working as best they can.

We have submitted policy proposals and recommendations on waiting times for women with symptoms of breast cancer in Wales and the monitoring of referral waiting times, ahead of the update to the Wales Cancer Delivery Plan and the Welsh Assembly elections. We have also continued to press the Welsh Government to publish the results and recommendations of the pilot undertaken to look at a new Single Cancer Pathway, which aims to ensure that all women with symptoms that could be breast cancer are seen as quickly as possible.

Breast Cancer Now has been an active supporter of the Scottish Government's 'Detect Cancer Early' programme. Breast cancer is one of three cancers being targeted by this campaign, which has led to an increase in the detection of early stage breast cancers by six per cent. Breast Cancer Now has campaigned to ensure that funding for this programme is sustained and that breast cancer remains one of the focus diseases. The Scottish Government has now committed to this programme in their new Cancer Strategy 'Beating Cancer: Ambition and Action'.

"This is a great leap in our understanding of DCIS, but it is also just the first step. We now hope to carry out a larger study assessing more genetic changes to discover whether there are any that predispose solely to DCIS, something that could help us predict whose non-invasive cancer is unlikely to progress."

Dr Elinor Sawyer,
King's College London and Guys and St Thomas' NHS Foundation Trust.



An extra-ordinary mother

“I’ll never forget the image of my darling Becs, jostling with her older brother and sister to give me her homemade Mother’s Day card.

“Becs was always our golden girl. Born two weeks late, and following a miscarriage scare, she arrived determined to live life to the full, no matter what. Over the years, she grew into a kind, loving young woman who filled us all with such joy.

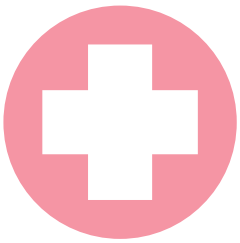
“We were devastated when, at just 32 years old, Becs was diagnosed with breast cancer. Treatment started immediately. Becs received the very best care from the doctors, and faced chemotherapy and a mastectomy with almost superhuman courage. But it was a gruelling regime that often left her sick and exhausted.

“It’s every mother’s nightmare to see their child in pain and not be able to take that pain away. Our nightmare only got worse when, just months later, we discovered the cancer had spread to Becs’ liver.

“The cancer spread aggressively and eventually could not be stopped. Becs died less than a year after being diagnosed with the disease.”

Amanda campaigns with Breast Cancer Now for improvements in treatment and care for breast cancer patients.

Treatment



Our aims and results in 2015-2016

We will advance understanding of the genetic faults that drive breast cancer and allow it to become resistant to treatments, which will lead to more personalised treatments.

We have awarded 11 new grants to researchers who are looking for new ways to treat breast cancer more effectively, prevent drug resistance and develop a better understanding of the different types of breast cancer to enable greater personalisation of treatments. Our newly awarded grants include a project to investigate how the non-cancerous cells that surround the tumour, known as the stroma, help the cancer cells to develop resistance to treatment.
Over the past year, our funding has enabled researchers to make many important findings, including the discovery of a signal known as NOTCH4 which allows breast cancer stem cells to 'hide' from treatment. Treating patients with drugs that block this signal may mean we can prevent resistance to treatment developing and so stop breast cancer returning.

We will ensure continued government investment in research – including in strategic areas of relevance to treatment, such as stratified medicine, genomics and biobanking.

We have continued to work in partnership with the Chief Scientists Office (CSO) in Scotland, with whom we are jointly funding a new project at the Beatson Institute in Glasgow. This project will investigate a promising new drug target for triple negative breast cancer.

As a member of the Association of Medical Research Charities (AMRC) we are working to highlight the importance of the Charity Research Support Fund in England and seek the continuation of the Fund and a commitment to increasing it in line with charity investment.

We are also an active member of the AMRC's EU working group, which is seeking to develop policies, lobby and campaign on the impact of the decision to leave the EU on medical research. So far, we have responded to the Science and Technology Committee inquiry into leaving the EU, both individually and as a member of the AMRC.

We will develop tests to predict how well patients will respond to therapy options and use imaging to monitor their response, ultimately ensuring each person gets the most effective treatment for them.

Our researchers have helped to evaluate a new test, known as Endopredict, which is used to predict the risk for individual patients of their breast cancer spreading to other parts of the body within 10 years of diagnosis. Endopredict was found to be more accurate than OncotypeDX, the approved test currently in use, and provided results more quickly, meaning that it could allow informed decisions about treatment to be made earlier.

We have also funded research into developing highly sensitive blood tests which can detect whether a patient has become resistant to hormone therapies, such as tamoxifen and aromatase inhibitors. This could allow patients to have their treatment rapidly switched to different drugs that are more likely to benefit them.

“It could help improve treatment for a large number of women with breast cancer by allowing doctors to better predict which women are least likely to go on to develop secondary cancer – and could therefore be spared from undergoing the chemotherapy often offered early on in treatment to reduce that risk.”

Dr Richard Buus, of the Breast Cancer Now Toby Robins Research Centre at The Institute of Cancer Research

We will ensure patients can access the most appropriate treatments that can offer them the best quality of life, that the UK (and EU) research environment is doing all it can to allow rapid translation of research into clinical practice and that the NHS fulfils its duties to promote and support research.

We have worked with leading experts to promote the results of a meta-analysis which indicated that over 1,000 lives can be saved every year by giving women with primary breast cancer a type of drug called bisphosphonates. We have modelled the cost of prescribing these drugs in each of the devolved nations and continue to use this to work with health authorities across the UK to ensure bisphosphonates are made routinely available to all eligible patients. We continue to collaborate with decision makers on this issue.

We continued working with Nick Thomas-Symonds MP to bring forward the Off-patent Drugs Bill 2015-16. The Bill called on the UK Government to seek a licence for repurposed, off-patent drugs, where they are supported by strong evidence but there is no commercial incentive for a pharmaceutical company to seek a licence. It is estimated that 60 to 70 MPs attended the Second Reading of the Bill in November 2015. Although the Bill fell at this stage, it resulted in a significant programme of work with the Department of Health, amongst others, to explore non-legislative solutions to improving the uptake of repurposed, Off-patent drugs where they are supported by strong evidence. We are continuing to work with the British Generic Manufacturers Association to develop a way for Off-patent drugs to be licensed for use.

In total, 9,252 members of the public emailed their MP in support of the Bill, which was also supported by four Medical Royal Colleges, the British Medical Association, NHS Clinical Commissioners, 12 national charities and 41 clinicians who signed a letter in the Daily Telegraph. The outcome of the debate received extensive press coverage.

The Scottish Government's Cancer Strategy contains a commitment to working to improve access to Off-patent drugs for new uses.

We have worked with breast cancer oncologists to analyse the results of a UK-wide survey about the use of bisphosphonates for the prevention of secondary breast cancer. In England there is a lack of clarity regarding who is responsible for commissioning these drugs, and clinicians have highlighted this as a barrier to prescribing them to patients who need them. We have therefore sought to highlight these problems, urging NHS England to clarify who is responsible.

We will continue to work closely with patients and partners across the NHS to improve breast cancer services and make sure that everyone living with breast cancer receives the best possible care and support.

Over the last year, Breast Cancer Now has continued to support the NCRI Breast Cancer Symptom Management Working Group. Membership and support of this group has allowed Breast Cancer Now to understand and offer assistance, where appropriate, on the biggest issues people living with and beyond breast cancer are facing. These range from debilitating hot flushes and night sweats to sexual dysfunction and isolation. Through this group we have commented on the NICE menopause guidelines to ensure that breast cancer patients are at the forefront, not only with regards to recommendations but also research priorities. The group also provides advice to researchers looking to apply for funding on breast cancer symptom management research, allowing us to have greater visibility as a funder of this type of research and to direct relevant applications towards Breast Cancer Now.
As part of our commitment to minimising the negative side effects of breast cancer treatment and ensuring patients' needs are met, we have recently funded research to investigate whether cognitive behavioural therapy, delivered by a nurse, can reduce the impact of side effects, such as hot flushes, on patients' quality of life.

"The Service Pledge for Breast Cancer programme helps breast cancer services turn local patient feedback into real service improvements, gathering patient feedback through surveys and workshops to inform service improvement plans. It has been shown to engage staff, improve patient experience and lead to time and cost savings."

"**Untapped potential:** Bringing the voluntary sector's strengths to health and care transformation." New Philanthropy Capital report on behalf of the Richmond Group

Since August 2015, we have worked with 23 hospitals across England to deliver our Service Pledge for Breast Cancer programme. The Service Pledge helps breast care services turn local patient feedback into real service improvements. We help hospitals involve their patients and staff to discuss, develop and implement an action plan of patient-centred improvements.

So far we have helped 13 of these hospitals to design improvements to their breast care services. We are also working with the other 10 hospitals to collect and review patient and staff feedback on their breast care services. This work has involved 65 patient advocates, trained to give an expert patient perspective, as well as local patient representatives and those attending workshops to discuss breast care services. A total of 81 improvements have been identified, 15 of which had been implemented as of April 2016.

In April 2016, we also agreed with the London Commissioning Support Unit to pilot a new approach to the Service Pledge programme, working with the three hospitals in the Barts Health NHS Trust that deliver breast care services. In addition to improving patient services, we are aiming to drive improvements at Trust and commissioning level, enabling greater cohesion and ultimately establishing the Barts Health NHS Trust as a world-leading breast cancer service.

We commissioned New Philanthropy Capital (NPC) in May 2016 to help us develop a theory of change for the Service Pledge programme. This will help us implement a more effective and streamlined evaluation framework to measure the impact of the Service Pledge programme on patient experience and benefits to the NHS. It will also help us identify how we can further develop our Service Pledge to maximise its impact.

Through our role on the National Cancer Patient Experience Survey Advisory Group, we can influence the key questions and standards of care and use this forum to highlight any barriers we perceive to be stopping patients from accessing the best possible treatment and care.

We are dedicated to ensuring that all those affected by breast cancer have the knowledge and support they need to make informed decisions about their care, and as such have developed a range of information on primary breast cancer that has reached over 1,200 people.

A determined husband

Dr Paul Shore has always been passionate about breast cancer, but when his wife Linda found a lump, his work suddenly took on a new meaning.

"I think Paul found it difficult because he knew what was coming next," said Linda.

"Sometimes he knew too much, but at the same time, it meant I was able to ask all the right questions, and it helped that Paul could explain everything to me."

But first, Linda and Paul faced the difficult task of telling their children, William and Beccy, aged just eight and six. "We knew my appearance would change after the mastectomy and chemotherapy, so I sat with them and read a kids' book called 'Mummy's Lump', which made it easier."

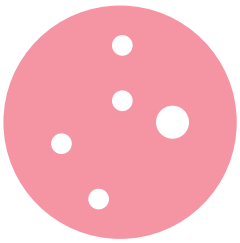
Thankfully, Linda's surgery went well, and she underwent chemotherapy. "It drained my energy. I'd plummet for a week, crawl back out and do it all over again. But it was never as bad as I thought it would be."

"It's been a difficult journey," Paul admits, "but my understanding of breast cancer meant that we knew where we were going, which helped us both. It has just shown me how important it is to pursue breast cancer research."

Linda's cancer is now under control, and the couple are enjoying family life.

Funded by Breast Cancer Now at the University of Manchester, Dr Shore's research will help to reveal how breast cancer cells spread to the bone. This could lead to new treatments to stop this from happening.

Secondary breast cancer



Our aims and results in 2015-2016

We will understand the processes that allow breast cancer to spread to different parts of the body and resist treatments.

Over the past year, we've focused on improving our understanding of how and why secondary breast cancer develops, and have awarded 11 new grants to further our understanding of this field. This includes developing new models with which to study secondary breast cancer and identifying the different molecules and signals that help secondary tumours develop.

Our researchers have made a crucial discovery about how cancer cells interact with and 'recruit' non-cancer cells to help the tumour grow and spread to other sites in the body. This may lead to the development of new drug targets and new approaches to treating and preventing secondary breast cancer.

We will find the most effective drug combinations and develop new treatments to prevent and control the spread of breast cancer, while making sure that patients' quality of life is at the fore.

Our researchers have continued to advance understanding of why breast cancer cells are able to spread to other parts of the body and form secondary tumours. They are using this knowledge to develop new ways to stop this happening. Using samples from the Breast Cancer Now Tissue Bank, our researchers are studying the biological mechanism and molecules which are required for breast cancer cells to spread, in order to find out whether targeting these factors can improve outcomes for patients.

We will continue to campaign for reform of the drug appraisal system and improve routine access to the most effective breast cancer treatments, regardless of price, for every woman in every part of the UK.

In October 2015, we petitioned pharmaceutical giant Roche to lower the price of Kadcyla, a life-extending drug used to treat secondary breast cancer, which was at risk of being removed from the Cancer Drugs Fund. Our petition received over 42,000 signatures and we presented it to Roche UK's General Manager, accompanied by a group of patient representatives. This led to Roche lowering the price of Kadcyla and making a deal with NHS England, which allowed the drug to remain available to patients.

We remain committed to ensuring breast cancer patients can access the drugs they need. We have responded to proposals from NHS England and NICE to reform the Cancer Drugs Fund, highlighting our concerns about the impact of current plans on patients. We also led the Cancer Drugs Fund Charity Coalition in writing an open letter to the Prime Minister to voice our fears for the future of access to cancer treatments and asking for a commitment to reform of NICE's drug appraisal processes.

"I have been on Kadcyla now for 18 months and my latest scan results showed no evidence of active cancer in the body at all and only scarring on the brain from two separate radiotherapy treatments that targeted two tumours in my brain. It shocks me to the core that this drug may not be available for others."

Julie Strelley-Jones, living with secondary breast cancer and being treated with Kadcyla

In collaboration with Prostate Cancer UK, we published a report comparing the drug appraisal systems and availability of breast and prostate cancer treatments in countries of similar wealth. As a result, we aim to be able to make recommendations for aspects of international systems that could be adopted in the UK.

In July 2016 we secured a Health Oral Parliamentary Question, asking the Secretary of State to review NICE's health technology appraisal process for cancer drugs. This followed a survey that we ran to gain a better understanding of patients' experiences of accessing, or hopes of accessing, treatments on the Cancer Drugs Fund and their views on the proposed new system. We also held a summit on access to drugs, which was attended by high-level stakeholders from the pharmaceutical industry, charities, academia, NICE and NHS England.

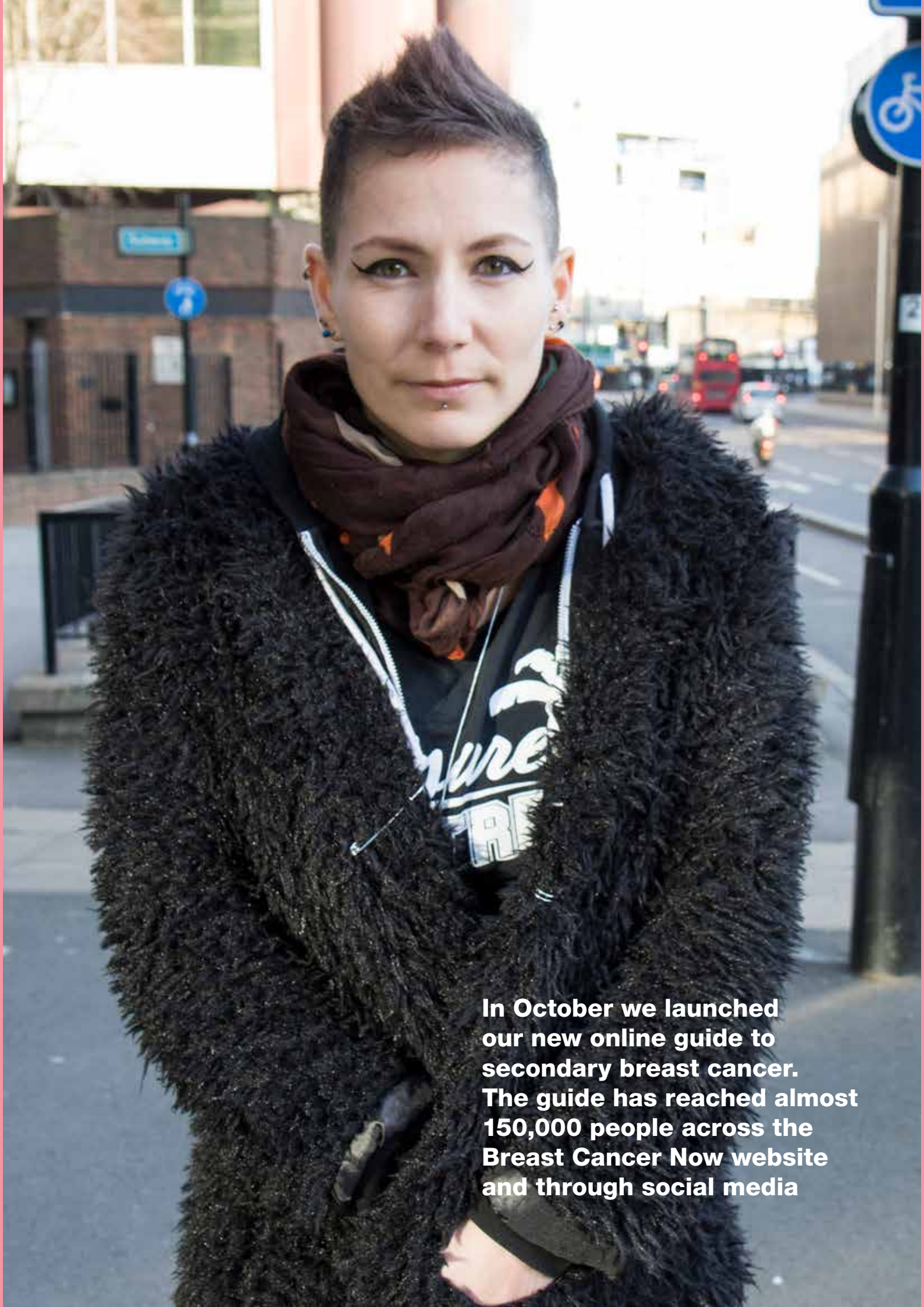
Breast Cancer Now is also leading efforts to reform the way patients access medicines in Scotland. On the back of our calls for change, the Scottish Government announced a new independent review of access to medicines in January 2016. We have produced a detailed plan to create systems that put people first. We continue to campaign for the review to adopt our plan as a way to overhaul access to medicines in Scotland.

We will improve the quality of healthcare and support for secondary breast cancer patients, making sure their needs are met.

Since August 2015, we have worked in partnership with Breast Cancer Care to deliver our Secondary Breast Cancer Pledge programme to 11 hospitals across England and Wales. This programme supports hospitals to review the standards of care they provide to people living with secondary breast cancer and identify ways to improve their services and the experience for patients. Surveys were sent to over 1,000 patients, of which almost 50 per cent responded. We spoke to 50 patients directly about their experiences, and suggestions for improvements to services, and worked with at least one patient representative at each hospital to facilitate discussions between hospital staff and patients to co-design improved services and care for people with secondary breast cancer. Since August 2015, a total of 46 improvements have been identified for five of the 11 hospitals, with the remaining six hospitals due to confirm and start making their improvements by December 2016.

In October 2015, we launched our new online guide to secondary breast cancer, a personalised resource for people affected by secondary breast cancer within the UK. The guide explains clearly and compassionately what patients can and should expect from their treatment and care.

To produce this guide we worked in collaboration with health experts and patients to provide high quality, accessible online information that users can tailor according to their own circumstances. This includes location within the UK, the type of breast cancer a user has and the area of the body to which it has spread. The guide also includes videos from breast care nurses and other women living with secondary breast cancer on a range of topics, from first noticing symptoms to ways to manage fatigue. Overall, the guide has reached almost 150,000 people across the Breast Cancer Now website and through social media, and has been viewed nearly 60,000 times.



In October we launched our new online guide to secondary breast cancer. The guide has reached almost 150,000 people across the Breast Cancer Now website and through social media

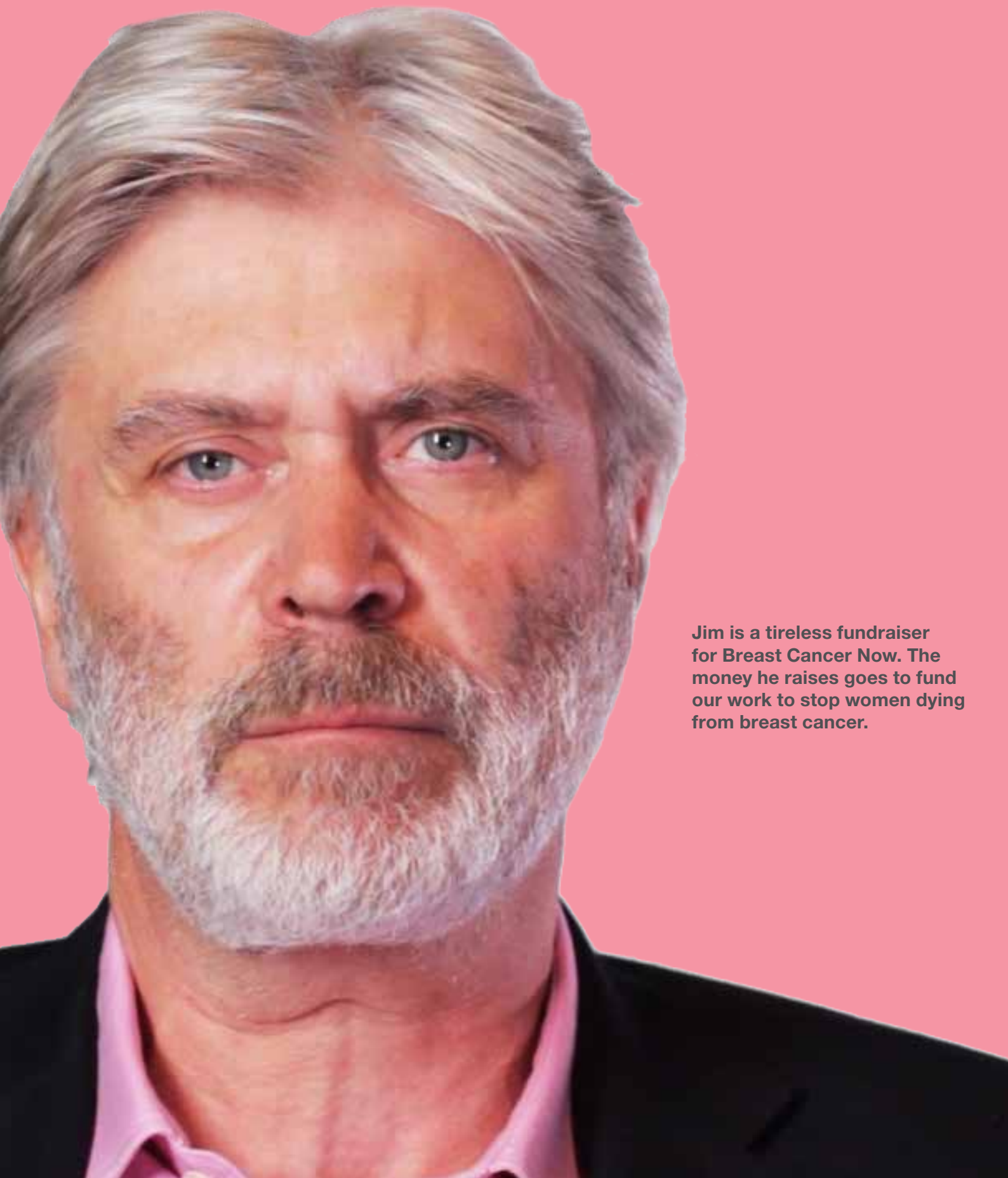
A brave father

“Nothing has ever been harder than telling the children that their mother was gone.

“After getting the diagnosis that it was breast cancer, my wife Moira had the lump removed and started treatment. The doctors were wonderful but the treatments were barbaric, and it was so hard to watch her suffer. But she got through it and seemed to be recovering well.

“Just six months later, we found out the cancer had spread - into her bones, lungs, liver and finally her brain. This is what’s known as secondary breast cancer. Those three little words are the diagnosis everyone living with this disease fears. It meant Moira’s breast cancer couldn’t be cured.

“Moira was surrounded by her family on the day she died – our three children, her father and her brother were all with her. That is the only positive thing I can take from that awful day: that she knew how much we loved her. She died in my arms that night.



Jim is a tireless fundraiser for Breast Cancer Now. The money he raises goes to fund our work to stop women dying from breast cancer.

Collaboration & innovation

Our aims and results in 2015-2016

We will actively seek out new partnerships and collaborations to radically speed up critical research into all aspects of the disease.

We previously heard from the scientific community that, in order to speed up progress in breast cancer research, they needed a platform to share their ideas and knowledge with other researchers from different disciplines. In July 2016, we held the first UK Breast Cancer Research Symposium, a two day conference that focused on translational research – taking discoveries from the lab and turning them into real progress for patients. The Symposium brought together more than 200 delegates from 11 countries, including breast cancer scientists from backgrounds as diverse as immunology, genetics and 3D tissue modelling, as well as clinicians, nurses, patient advocate groups and representatives from the pharmaceutical and biotechnology industry. This event helped to foster collaborations between these different groups, advance our understanding of breast cancer and shape how we tackle the critical issues facing people with the disease.

We will work in partnership and collaboration with other organisations, researchers and healthcare professionals to promote improvements in treatment and care.

Last year we launched the Breast Cancer Now Catalyst Programme, an exciting new initiative designed to bring together leading researchers, pharmaceutical companies and funding bodies. Through collaboration and the pooling of resources, we aim to accelerate the development of treatments and secure better outcomes for patients.

Our first Catalyst Programme collaboration has been made with Pfizer, which has committed £10 million to funding translational research over three years. Pfizer has also granted researchers independent access to over 13 existing and experimental drugs in their oncology portfolio, ensuring intellectual integrity and researcher independence remain at the heart of the collaboration.

> "The Breast Cancer Now Catalyst Programme places NHS patients at the heart of 21st Century research collaboration and promises to bring innovative new treatments to patients as fast as possible.
>
> **George Freeman MP**, Minister for Life Sciences

The collaboration will allow the scientific community to propose research projects based on their unique insights into patient needs and understanding of cancer therapies. Proposals will be reviewed by an independent review committee and external peer reviewers, with the successful proposals receiving both funding and access to Pfizer's drug pipeline. Support for the Breast Cancer Now Catalyst Programme was given by the then Minister for Life Sciences, George Freeman MP.

Our work promotes collaboration and innovation. These aims and results support our work across risk and prevention, early detection and diagnosis, treatment and secondary breast cancer.

Over the past year we have formed vital relationships with MPs across the UK, with 211 MPs and 18 members of the Welsh Assembly signing up to become Breast Cancer Ambassadors.

"Too many people lose loved ones to this disease. Soon we will be publishing the Scottish Government's cancer strategy and I can confirm that it will reflect our commitment to the 2050 ambition. Scotland can and will lead the way in stopping deaths from this terrible disease."

First Minister, **Nicola Sturgeon**

In March 2016, every mainstream political party in Scotland got behind Breast Cancer Now's 2050 Challenge campaign. This means that the Scottish Government and all parties in the Scottish Parliament are united in helping us to achieve our ambition that by 2050, everyone who develops breast cancer will live. Breast Cancer Now's campaign also led to our 2050 ambition being adopted by the Scottish Government as a central part of their Cancer Strategy.

We have continued to provide the Secretariat to the All-Party Parliamentary Group on Breast Cancer which has included the organisation of a number of roundtable discussions with high-level stakeholders, such as the then National Clinical Director for Cancer, Professor Sean Duffy.

We have also continued to provide the Secretariat to NHS England's Breast Cancer Clinical Reference Group and worked on the development of breast cancer service guidance and examples of breast cancer pathways. These aim to support the commissioning of gold standard breast cancer services in England and are now awaiting publication.

As the Secretariat to the Cancer Drugs Fund Charity Coalition and the Off-patent Drugs Charity Coalition, we ensure we are working with other charities with similar interests to maximise our impact and improve access to drugs for cancer patients.

We are an active member of the Department of Health and AMRC roundtable on repurposed drugs and have been pivotal in proposing a pathway through which repurposed drugs can be routinely commissioned.

We have also been successful in gaining a patient representative role on NICE's guideline development committee for primary and locally advanced breast cancer, and have retained our role on NHS England's Chemotherapy Clinical Reference Group and the Advisory Committee on Breast Cancer Screening.

We have taken an active role in the Scottish Cancer Coalition and the Scottish Cancer Prevention Network, and work closely with clinicians via our Scottish Advisory Panel. As a result, we have been able to contribute to reviewing the Scottish Cancer Strategy and NHS Scotland referral guidelines, and assessing quality performance indicators.

We aim to ensure that all of our health information is accurate and up to date, giving those affected by breast cancer the knowledge and confidence they need to make informed decisions about their health and care. When developing new materials, we work closely with members of the public to understand what information they wish to see and ensure this information is clear and accessible to all. We are also guided by policy experts and have all materials reviewed by experts in the field. Our public health materials have gained the NHS England's Information Standard accreditation, a mark of quality.

We are a member of the Association of Medical Research Charities (AMRC), who audit the peer review processes of its member charities every five years. Peer review is a way of assessing the quality of scientific ideas and we use this process to select the best research to fund. We ask independent scientific experts to examine the research to see if it is important, well designed and practical. We passed the AMRC's 2015 audit and have been awarded a certificate to show that we follow best practice when peer reviewing our grant applications. The AMRC assessed the accountability, balance, independence, rotation and impartiality of our peer review process. This audit allows us to demonstrate our commitment to the highest standards of accountability and probity to donors, funders, the government and researchers.

We will ensure all of our work is informed by people affected by breast cancer and use their experiences and concerns to create change by keeping the disease at the top of the agenda.

In April 2016, we launched a pilot 'insight group' online, to investigate a new way of gathering feedback and offering a way for those affected by breast cancer to help shape and improve our policy and campaigns work, as well as providing insight to the wider organisation. We have also run surveys and focus groups and held meetings to bring our campaigners together to help us understand the motivations behind support for Breast Cancer Now and find out what our supporters believe our campaign priorities should be.

In July 2016, we launched the Big Breast Cancer Conversation, a project designed to help us fully understand the experiences of people affected by breast cancer. We will be talking to a wide range of women to find out how breast cancer has affected them, their friends and families, allowing us to gain a comprehensive view of the different experiences, views and opinions on breast cancer across the UK. This will, we hope, ensure the work of Breast Cancer Now reflect's the needs of these women, in terms of the research we fund, the services we provide and the ways our supporters can get involved.

Due to the differences in the NHS between the devolved nations, it is essential that we understand how breast cancer is experienced in Scotland, from diagnosis to treatment and care, and allow this to shape the way we work. That's why we have connected with and supported a wide range of people who have been affected by breast cancer in Scotland, and will continue to do so.

We are also working to develop a new organisational strategy, due to launch in the coming year. We have been consulting widely with scientific and policy experts and advisors to understand how Breast Cancer Now can have as great an impact as possible. When combined with the views and experiences of those affected by breast cancer, we will be able to define a new strategy that will help us work towards our ambition that, by 2050, everyone who develops breast cancer will live.

In July 2016, we launched the Big Breast Cancer Conversation

A project designed to help us fully understand the experiences of people affected by breast cancer.

Money matters

We have a huge ambition: to stop women dying of breast cancer by 2050 and we can't deliver without our generous supporters.

So, we're committed to ensuring the money our supporters donate to Breast Cancer Now delivers the greatest impact for women and men with breast cancer. Following our merger, we've been working hard to find savings and invest them back in our future work and will continue to do so.

We're also determined to be open and honest and our aim is to be as efficient as possible. So when you give us a £1 we aim to spend:

- **Over 60p** on work to stop women dying of breast cancer.
  - Of this, at least 50p will be spent purely on funding the very best breast cancer research in the UK and Ireland.
  - 10p will be spent on raising awareness of the disease – particularly the signs and symptoms that people need to look out for. And campaigning to make sure breast cancer patients get the best treatments and services available.
- **30p** will represent Fundraising costs. Of course, breakthroughs in breast cancer research take decades of effort from our scientists and this requires long term investment. Each year, this requires us to use some income to invest in raising more money in future. We focus on investing in the areas of fundraising that will produce the best long term returns. Currently the amount of legacy donations we receive is small, so presently our fundraising costs as a proportion of our funds, may look higher than some other charities.
- **10p** will represent our infrastructure costs that include our offices and our IT, finance and HR teams who make sure we have the skills, tools and information we need to be as efficient as we can. These costs also ensure we have strong governance across the organisation and support the Board of Trustees in ensuring we spend our money on our charitable purpose and that we have a strategy that ensures we achieve our ambition to stop women dying of breast cancer by 2050.

OLYMPUS

What does the future look like? Our next steps...

In 2015 we published our Action Plan for Breast Cancer, which sets out the key challenges we must overcome if we're to achieve our ambition of stopping women dying from breast cancer. These challenges underpin everything we do and our approach to tackling this devastating disease.

Risk and prevention

The best weapon we could have against breast cancer is the ability to stop it occurring in the first place. To do this, we need to know who is most at risk of developing breast cancer and what we can do to prevent it.

We will

- Understand how genetic, hormone and lifestyle factors lead to the development of breast cancer, and use this knowledge to accurately assess an individual's personal risk of the disease
- Identify sustainable lifestyle changes, such as diet and physical activity which can reduce the risk of breast cancer, and help women understand how making these changes could benefit them
- Find the best risk-reducing interventions for women at increased risk of breast cancer, and ensure these are made available and used as effectively as possible
- Ensure that the policy and public health environment responds to the latest evidence to deliver effective risk and prevention tests and treatments, as well as policies to support healthy, active lifestyles and improved public health in order to prevent more breast cancer cases.

Early detection and diagnosis

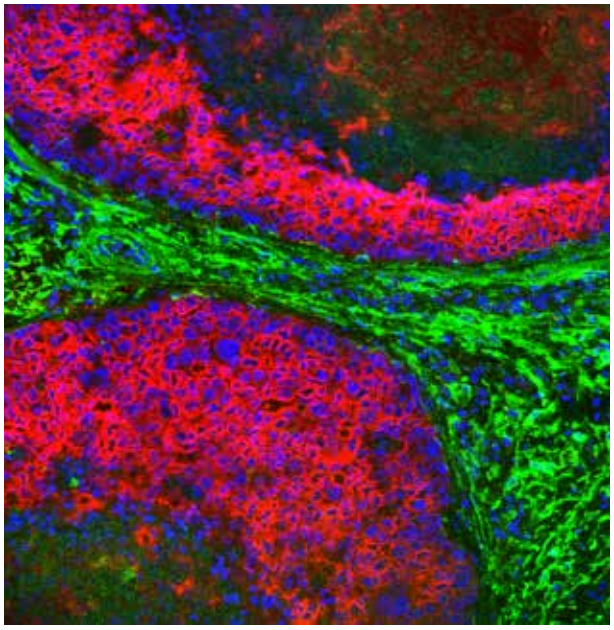
The earlier breast cancer is found and the earlier treatment is started, the greater the chances are of surviving the disease. Breast awareness and breast screening are vital ways to ensure women are diagnosed early.

We will

- Find new ways to detect breast cancer at its earliest stages and ensure that this knowledge is implemented
- Monitor developments in the stage at which breast cancer is detected across different areas and groups, and raise awareness of the signs and symptoms of breast cancer, giving women the confidence to take any concerns to their doctor as quickly as possible
- Develop better methods of differentiating between very early breast cancers that require treatment and those that don't, avoiding over-diagnosis and over-treatment
- Work with government, clinicians and the NHS to improve earlier diagnosis and detection by monitoring and influencing the quality of breast cancer services, including GP referral pathways, to ensure they're working as best they can.

Treatment

Breast cancer is not just one disease – there are more than 20 different types of breast cancer that we know of. That's why treatments must be highly targeted, personalised to each unique individual and their particular form of breast cancer. But there's more we need to know before we can truly fit the right treatment to the right person.



We will

- Advance our understanding of the genetic faults that drive breast cancer and allow it to become resistant to treatments, which will lead us to more personalised treatments
- Ensure continued government investment in research – including in strategic areas of relevance to treatment, such as stratified medicine, genomics and biobanking
- Develop tests to predict how well patients will respond to therapy options and use imaging to monitor their response, ultimately ensuring each person gets the most effective treatment for them
- Ensure patients can access the most appropriate treatments that can offer them the best quality of life, that the UK (and EU) research environment is doing all it can to allow rapid translation of research into clinical practice and that the NHS fulfils its duties to promote and support research
- Continue to work closely with patients and partners across the NHS to improve breast cancer services and make sure that everyone living with breast cancer receives the best possible care and support.

Secondary breast cancer

Secondary breast cancer – breast cancer that's spread to other parts of the body – kills nearly 1,000 UK women a month. We will fight to change this by ramping up research efforts to stop women dying from the disease, and work to ensure that women living with secondary breast cancer receive the best possible care.

We will

- Understand the processes that allow breast cancer to spread to different parts of the body, and those that allow secondary breast cancer to resist treatments
- Find the most effective drug combinations and develop new treatments to prevent and control the spread of breast cancer, while making sure that patients' quality of life is at the fore
- Continue to campaign for reform of the drug appraisal systems, and improve routine access to the most effective breast cancer treatments for every woman in every part of the UK
- Improve the quality of healthcare and support for secondary breast cancer patients, making sure their needs are met.

Collaboration & innovation

The time has come to develop a more agile response to breast cancer. In order to get ahead of this complex disease, we must work in new, collaborative ways. Our impact relies on our ability to translate all of our work into tangible benefits for those affected by breast cancer.

We will

- Actively seek out new partnerships and collaborations to radically speed up critical research into all aspects of the disease
- Work in partnership and collaboration with other organisations, researchers and healthcare professionals to promote improvements in treatment and care
- Ensure all of our work is informed by people affected by the disease and use their experiences and concerns to create change by keeping breast cancer at the top of the agenda.

Our work promotes collaboration and innovation. These next steps support our work across risk and prevention, early detection and diagnosis, treatment and secondary breast cancer.

Financial review

One year on from the merger of Breast Cancer Campaign and Breakthrough Breast Cancer we are demonstrating that by uniting to stop breast cancer taking lives, we are already having a truly positive impact for people affected by the disease across the UK.

This report reflects the first full year of activities of the merged charity Breast Cancer Now. On 31 March 2015, the activities, assets and liabilities of Breakthrough Breast Cancer and Breast Cancer Campaign were transferred to Breast Cancer Now. The cost base of the organisation has been significantly reduced, with support costs dropping £2.4m (41%) and the direct costs of delivering charitable activities dropping £0.7m (16%).

Income
During the year, total income increased from £27m to £27.6m with the main sources of income continuing to be individual giving and corporate partnerships. Income from charitable activities increased due to royalties generated from intellectual property from Breast Cancer Now-funded research activities.

Support from corporate partners grew by more than £2m to over £8m, with M&S, Asda and Debenhams providing fantastic support. Fashion Targets Breast Cancer, our flagship partnership with the fashion and retail sector, continued to perform strongly in its 20th year, generating £860,000.

Over 6,000 people ran, cycled, trekked or jumped out of planes for Breast Cancer Now, raising over £2m. Our partnership with 12 student RAGs raised over £450,000, making us one of the leading student fundraising charities. Our community supporters raised over £2.2m from cake sales, quiz nights, gala balls and a huge range of other activities. Our 63 local groups raised vital funds and helped promote our breast cancer awareness messages.

Over 50,000 individuals generously supported us through regular direct debit, cash or lottery donations. Their committed support enables us to plan for the future and provide the continued funding which is so important to the scientists we work with. Wear it pink, our fundraising mass participation event where people dress up in pink at work, home or school, raised over £1.9m

Once again, we were hugely fortunate to benefit from Walk the Walk, with a donation of £1.3m

Our Special Events programme had another successful year, with our fantastic volunteer committees of the Bigger Bounce gala ball raising £1m and the Pink Ribbon Balls in London and Manchester raising £600,000.

Over the coming year, we will be continuing to put in place plans to provide the growing and sustainable income base the charity needs to deliver its vision.

Expenditure on fundraising during the year declined from £14m to £13m (7%) due to efficiencies identified as a result of the merger. Fundraising costs include communications costs relating to fundraising activity.

Charitable expenditure
Charitable spend, which includes all activities relating to research, policy and campaigning, and public health and information, increased from £16.3m to £18.2m (12%). This increase in investment in our work into risk and prevention, early detection, treatment and secondary breast cancer includes a 32% increase in research grants in the year, from £9.9m to £13.1m

Merger costs
2015-16 was a year of consolidation and driving improvements in the new charity. There were merger costs of £0.3m during the year relating to residual brand work for the new charity and terminating contracts relating to former property obligations.

Reserves
The charity holds reserves to provide funding for future research commitments and a contingency against unforeseen operational cost pressures or reduction in income. Each year the Trustees review reserves levels in light of the year ahead to ensure there are adequate funds to support the organisation.

Reserves are held in two types:

- Restricted funds arise as a result of the donor making a specific stipulation as to how the funds may be used. This is often relating to a specific activity or grant that may fall over a number of year and therefore the balance carried forward at the year end is committed for those activities. Restricted funds stood at £1.17m at the year end.
- Unrestricted funds arise when no stipulation is made by the donor. The Trustees are responsible for ensuring these funds are spent in line with the charity's objectives in a timely fashion. Unrestricted funds excluding those held in fixed assets of £1.44m stood at £8.6m at the year end.

During the year, the Trustees considered that a sufficient level of reserves for the organisation would be a minimum of three months of operating costs. The current reserves position is in line with policy. Total funds at the year end were £11.2m.

Financial statements
The charity's consolidated financial statements are set out on pages 53 to 68, including the results of the charity's subsidiaries. Five of the six subsidiaries are wholly owned and the charity has two thirds ownership of Pink Ribbon Ltd. Breast Cancer Care owns the remaining third. Details are included in note 11 to the financial statements.

The financial results of the charity's regional groups are included within the consolidated results, as they operate within the same charity registration and are governed by Breast Cancer Now's regional group constitution.

Going concern
The Trustees have reviewed the charity's financial position, future plans and internal processes, and consider that the charity has adequate resources and infrastructure to continue operating for the foreseeable future. As a result they continue to adopt the going concern basis of accounting in the preparation of the annual accounts.

Grants
Breast Cancer Now funds research of the highest quality. We support the scientific community across the UK through multiple funding mechanisms, through our Tissue Bank and by supporting conferences and training courses.

Some of our funding supports centres of excellence, as we have taken a strategic decision to build a critical mass of research in a small number of locations. These are based in higher education institutions and close to hospitals and breast units, working under one roof in integrated, multidisciplinary programmes of research. The prime example of this is our Breast Cancer Now Toby Robins Research Centre in London, where more than 100 researchers are examining and collaborating on many different aspects of breast cancer research.

We also run an open grant funding scheme, which supports all types of breast cancer research across the UK and Ireland from basic, translational and clinical to psychosocial research. This scheme is also the route by which we fund Fellowships and PhD students. This allows us to build a talent pool to ensure that the capacity for future breast cancer research in the UK remains strong.

All of these grant proposals undergo independent, external scientific peer review to ensure scientific excellence. Our peer review processes are in line with the guidelines of the Association of Medical Research Charities.

Over 6,000 people ran, cycled, trekked or jumped out of planes for Breast Cancer Now, raising over £2m.

Governance, structure and management

The Board of Trustees of Breast Cancer Now presents its annual report and accounts for the year ended 31 July 2016. These comply with the Companies Act 2006, Breast Cancer Now's governing document the Memorandum and Articles of Association, and Accounting and Reporting by Charities: Statement of Recommended Practice applicable to charities preparing their accounts in accordance with the Financial Reporting Standards applicable in the UK and Republic of Ireland (FRS 102) (effective 1 January 2015)

Status
Breast Cancer Now is a company limited by guarantee, governed by Articles of Association, registered in England and Wales (number 9347608) and registered as a charity in England and Wales (number 1160558), Scotland (SC045584) and Isle of Man (number 1200). The Trustees listed on page 45 are also the charity's Directors and have overall responsibility for the strategic direction and effective governance of the charity. The Trustees met regularly during the year.

The legal and administrative details are listed on page 69.

Objects
The charity's objects are to:

- Advance health, including the health of individuals suffering from cancer, patients receiving treatment, those convalescing following treatment by:
  - promoting activities which will assist with awareness, identification and prevention of cancer;
  - assisting those individuals who are disabled, infirm or in need of assistance;
- Advance the health of individuals by undertaking research into the causes and treatment of cancer on terms that the results of such research are published
- Advance public education in and understanding of the nature of cancer and its treatments, particularly (without prejudice to that generality) among sufferers of cancer and the families, friends and carers of such persons
- Promote such charitable objects concerned with medical research or the relief of sickness as the Directors shall in their absolute discretion determine.

Trustee recruitment, appointment, induction and training
Trustees are appointed, by the board, for an initial period of three years that can be extended for a further three-year term. A further one, two or three year term can be agreed between the individual and the chairperson, or three other directors if deemed to be in the charity's best interest.

Trustees contribute their services voluntarily and upon appointment will participate in a formal induction programme, including attendance at Breast Cancer Now events, visits to our offices and tours of our research units.

A skills audit of the Board is performed periodically to ensure the appropriate range of skills and expertise, including in areas of scientific knowledge, organisational strategy and management.

Board of Trustees
Members of the Board during the year were:

Professor Lynne Berry[3] **Chair**
Professor Trevor J Powles[4] **Vice Chair**
Caroline Mawhood[1,3] **Honorary Treasurer** – resigned 29 September 2016
Pascale Alvanitakis-Guely[1]
Dr Veronique Bouchet[4]
Professor Robert Coleman[4]
Elizabeth Doogan-Hobbs[1] – resigned 29 September 2016
Professor Gareth Evans[4]
Professor Adrian L Harris[4]
Nicholas Hazell[1,2] – resigned 19 February 2016
Susan Johnson[2]
Ruth Plowden[4]
Jennifer Price[2,3] – resigned 25 November 2015
Laura Simons[3]
Professor Hilary Thomas[3,4] – resigned 29 September 2016
Susan Gallone[1] – appointed 29 September 2016

[1]member of the finance and investment committees
[2]member of the risk committee
[3]member of the remunerations, appointments and nominations committee
[4]member of the science strategy committee

Chief Executive and management
Day to day management of the charity is delegated to the Chief Executive assisted by the Executive Leadership Team, other staff and volunteers. The Board approves the delegation of financial authority through the Chief Executive to the staff with specific limits imposed within an approved scheme of delegation.

Executive Leadership Team (at 31 July 2016)
Delyth Morgan (Chief Executive)
Mary Allison (Director for Scotland)
Fiona Hazell (Director of Communications & Engagement)
Polly McGivern (Director of Finance & Corporate Services)
Catherine Miles (Director of Fundraising)
Dr Simon Vincent (Interim Director of Research)

During the year the Executive Leadership Team (ELT) were regarded as the charity's key management personnel per FRS102. Total earnings, including pension contributions, received by members of ELT during the year were £564k (14/15 £1.02m). The Chief Executive, the highest paid member of staff, received a salary of £117,160 for the year.

Staff Employee reward
In 2015-16, Breast Cancer Now employed 166 staff, but we rely on our entire workforce, including staff, supporters, volunteers and trustees, and their amazing commitment, dedication and support to do the work we do.

Our Remuneration, Appointments and Nominations Committee agrees and monitors Breast Cancer Now's reward strategy, pay and benefits policies and annual pay review process, all of which determine how pay levels are decided. We are committed to equality in our pay and benefits policy, and aim to ensure that our levels of pay reflect the knowledge, skills, experience and competencies of our staff. We aim to pay salaries at the median level of the voluntary sector, and we regularly use salary survey data to check that our pay remains in line with the market we operate in. The Committee approves the annual pay review process, which is determined partly by average pay awards across the sector, and partly by the charity's financial performance to fund an annual cost of living increase.

Executive pay is governed by the same rules and review processes as for all other staff, and we offer the same level of benefits to the executive team as to the rest of our staff. Jobs are evaluated and graded into a framework with pay bands that are transparent and published to all staff.

Trustees freely give their time and do not receive payment for the work they do, other than travel expenses.

Information
We share important and relevant information with our staff through our intranet, as well as through team briefings, and informal monthly face to face meetings to update staff on important news, achievements and upcoming events. We carry out an annual all-staff opinion survey to gather staff feedback, and use focus groups, workshops and smaller surveys to engage staff in our work.

Equality
Breast Cancer Now acknowledges that everyone is unique with individual skills, knowledge and life experiences and everyone can make a valuable and positive contribution to the aims, values and strategic goals of the charity. We recognise the benefits of employing and engaging with individuals from all backgrounds and community groups as this helps build a workforce and business where creativity and valuing difference in others thrives.

We are committed to a policy that ensures all current and potential staff members and volunteers are offered the same opportunities regardless of their sex, sexual orientation, age, disability, gender status (in cases of gender re-assignment), pregnancy and maternity status, marital status, race and religion and belief (protected characteristics) and membership of or activities as part of a trade union, or social or economic status.

Fundraising ethos
Breast Cancer Now aims to build strong, respectful and empowering relationships with our financial supporters. It is only through the generosity of individuals, companies and organisations that we are able to fund the research which will stop people dying from breast cancer.

We believe people should feel inspired to give to or raise money for Breast Cancer Now, feel valued and respected by the charity and understand the life-saving difference their support makes. People should never feel harassed or pressured to give.
The following principles guide our fundraising strategy and operations:

- We aim to inspire people to give or raise money for Breast Cancer Now
- We thank supporters appropriately and demonstrate the difference their money makes
- We keep supporters' data secure and do not sell or share it for marketing purposes
- We manage agencies working for us closely and demand high standards of them to ensure supporters and the wider public do not feel pressured to give and are treated with respect at all times, with a particular focus on the protection of vulnerable people
- We listen to supporters and act on their communication requests

We endeavour to build long-term relationships with our supporters, enabling them to support the charity in all the different ways they choose. In turn, we know that for our supporters their charitable giving is an important and positive part of their lives.

Breast Cancer Now was a member of the Fundraising Standards Board during 15/16 and will be joining the new Fundraising Regulator. Breast Cancer Now's fundraising activities and compliance with fundraising regulations and best practice are closely scrutinised by our Internal Audit and Risk Committee, which in turn reports to our Board of Trustees.

Advisory boards and committees

Finance Committee
The Finance Committee is responsible for advising the Board on short and long-term financial planning, including review of financial plans, budgets and proposals, as well as playing a governance role with the review of financial policies, processes and controls. The Finance Committee met six times during the year.

Investment Committee
The Investment Committee is responsible for setting and recommending the investment strategy to the Board for approval and for overseeing the management and performance of investments. The Investment Committee met twice during the year.

Risk Committee
The Risk Committee sets the risk management process and ensures that strategic risks are identified, reported to the Board and, where necessary, brought to the attention of other committees, such as the finance and investment committees. The Committee ensures that risk priorities and relevant actions are highlighted to the Executive Leadership Team for further action by their directorates. The Committee also oversees any internal audit programme and ensures recommendations are implemented. The Risk Committee met four times during the 2015/16 operational year.

Remuneration, Appointments and Nominations Committee
The Remuneration, Appointments and Nominations Committee has delegated responsibility from the board for pay levels and performance appraisal methods across the organisation and appointment of Trustees, Fellows and Senior Executives including the Chief Executive. The Remuneration, Appointments and Nominations Committee met five times during the year.

Science Strategy Committee
The Science Strategy Committee is responsible for making recommendations to the Board as to which research projects should be funded and plays a key role in creating the research strategy. The Science Strategy Committee met three times during the year.

Risk
The Trustees reviewed Breast Cancer Now's key risks and are satisfied that risk management has been undertaken appropriately and that adequate systems were in place to manage risk. The Executive Leadership Team are responsible for the day to day risk management and ensuring that each directorate is aware of, and appropriately manages, their risks. The risk register was reviewed by the Risk Committee during the period.

The key risks facing the charity in the year ahead and mitigations to manage them, are in the table on the next page.

Risks	Mitigations
Attracting, developing and maintaining a skilled staff team to deliver our ambitious objectives and ensuring we are an employer of choice within the sector.	Continued investment in staff training and development including new management and leadership training; a review of staff pay and benefits to ensure competitive but appropriate pay levels; introducing a new well-being at work programme.
Changes in relevant policy, spending and practice, in particular in relation to the charity's research portfolio.	Policy landscape and research environment. Addressed through our influencing work including membership of relevant bodies and groups focused on research and health policy and practice across the UK. This includes groups established to assess and manage the impact of Brexit on medical research.
Developing strong brand awareness and reputation and a suitably diversified and competitive income stream, to ensure a stable foundations, in light of a challenging fundraising environment.	Ongoing investment in building awareness and understanding of the charity and its work alongside a broad portfolio of income streams for the short and longer term. Regular review of our fundraising practices and agencies to ensure up-to-date compliance and best practice. Membership of the relevant fundraising bodies and working groups across the UK.
Effectively managing activity and events connected to the charity that could impact reputation, trust or operations including issues relating to data security breaches, fundraising practices or the integrity of our research work.	Independent, external peer review processes to ensure scientific excellence of our research which passed the AMRC's 2015 audit; evidence-based policy and communications work, informed by and developed with patients. All health information carries NHS England's Information Standard accreditation. Member of the Fundraising Standards Board. On-going improvements made to data protection policies and practices, crisis management processes and IT security throughout the year.

Investments
Breast Cancer Now holds investments primarily in equities and bonds in accordance with the Investment Strategy approved by the Trustees.

The charity seeks to maximise the returns on investments, whilst appropriately safeguarding the capital, in line with the specific requirements set out in the Investment Strategy and have a moderate to low risk appetite with a desire for a balanced portfolio.

The performance of the investment portfolio in the year was in line with expectations.

Subsidiary Trading Companies
The charity has a number of trading subsidiaries that are detailed in note 11 of the financial statements along with their results for the year. The companies are all wholly owned subsidiaries, except for Pink Ribbon Ltd which is jointly owned with Breast Cancer Care. These subsidiaries carry out activities such as sub-licensing the charity's logo and the Fashion Targets Breast Cancer logo to commercial partners and delivering certain charitable partnerships. They transfer their profits to the charity under the Gift Aid scheme and their financial statements are consolidated into those of the charity.

Statement of Trustees' Responsibilities

The Trustees (who are also Directors of Breast Cancer Now for the purpose of company law) are responsible for preparing the Trustees' Annual Report, the Strategic Report and the financial statements in accordance with applicable law and United Kingdom Generally Accepted Accounting Practice (United Kingdom Accounting Standards).

Company law requires the Trustees to prepare financial statements for each financial year. Under company law the Trustees must not approve the financial statements unless they are satisfied that they give a true and fair view of the state of affairs of the charitable company and of the incoming resources and application of resources, including the income and expenditure of the charitable company for that period.

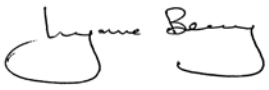
In preparing these financial statements, the Trustees are required to:

- Select suitable accounting policies and then apply them consistently
- Observe the methods and principles in the Charities SORP
- Make judgements and estimates that are reasonable and prudent
- State whether applicable UK accounting standards have been followed subject to any material departures disclosed and explained in the financial statements
- Prepare the financial statements on the going concern basis unless it is inappropriate to presume that the charitable company will continue in business.

The Trustees are responsible for keeping adequate accounting records that are sufficient to show and explain the charitable company's transactions, disclose with reasonable accuracy at any time the financial position of the charitable company and enable them to ensure that the financial statements comply with; the Companies Act 2006; the Charities Act 2011; the Charities (Accounts and Reports) Regulations 2008; the Charities and Trustee Investment (Scotland) Act 2005; the Charities Accounts (Scotland) Regulations 2006 (as amended); and the provisions of the Charity's constitution. They are also responsible for safeguarding the assets of the charitable company and hence for taking reasonable steps for the prevention and detection of fraud and other irregularities.

Insofar as each of the Trustees of the charitable company at the date of approval of this report is aware, there is no relevant audit information (information needed by the charitable company's auditor in connection with preparing the audit report) of which the charitable company's auditor is unaware. Each Trustee has taken all of the steps that he/she should have taken as a Trustee in order to make himself/herself aware of any relevant audit information and to establish that the charitable company's auditor is aware of that information.

This Annual Report, prepared under the Charities Act 2011 and the Companies Act 2006, was approved by the Board of Trustees of Breast Cancer Now on 15 December 2016, including in their capacity as company directors approving the Strategic Report contained therein, and is signed as authorised on its behalf by:



Professor Lynne Berry
Chair of Trustees

Independent Auditor's Report to the Members and Trustees of Breast Cancer Now

We have audited the financial statements of Breast Cancer Now for the year ended 31 July 2016 which comprise the Group Statement of Financial Activities, the Group and Company Balance Sheets, the Group Cash Flow Statement and the related notes numbered 1 to 22.

The financial reporting framework that has been applied in their preparation is applicable law and FRS 102, The Financial Reporting Standard applicable in the UK and Republic of Ireland. This report is made solely to the charitable company's members, as a body, in accordance with Chapter 3 of Part 16 of the Companies Act 2006 and to the charitable company's trustees, as a body, in accordance with section 44(1c) of the Charities and Trustee Investment (Scotland) Act 2005. Our audit work has been undertaken so that we might state to the charitable company's members those matters we are required to state to them in an auditor's report and for no other purpose. To the fullest extent permitted by law, we do not accept or assume responsibility to anyone other than the charitable company and the company's members as a body, for our audit work, for this report, or for the opinions we have formed.

Respective responsibilities of Trustees and auditor
As explained more fully in the Statement of Trustees' Responsibilities, the Trustees (who are also the directors of the charitable company for the purpose of company law) are responsible for the preparation of the financial statements and for being satisfied that they give a true and fair view.

We have been appointed as auditor under section 44(1c) of the Charities and Trustee Investment (Scotland) Act 2005 and under the Companies Act 2006 and report in accordance with regulations made under those Acts. Our responsibility is to audit and express an opinion on the financial statements in accordance with applicable law and International Standards on Auditing (UK and Ireland). Those standards require us to comply with the Auditing Practices Board's Ethical Standards for Auditors.

Scope of the audit of the financial statements
An audit involves obtaining evidence about the amounts and disclosures in the financial statements sufficient to give reasonable assurance that the financial statements are free from material misstatement, whether caused by fraud or error. This includes an assessment of: whether the accounting policies are appropriate to the charitable company's circumstances and have been consistently applied and adequately disclosed; the reasonableness of significant accounting estimates made by the trustees; and the overall presentation of the financial statements.

In addition, we read all the financial and non-financial information in the Strategic Report and the Trustees' Annual Report to identify material inconsistencies with the audited financial statements and to identify any information that is apparently materially incorrect based on, or materially inconsistent with, the knowledge acquired by us in the course of performing the audit. If we become aware of any apparent material misstatements or inconsistencies we consider the implications for our report.

Opinion on financial statements
In our opinion the financial statements:

- give a true and fair view of the state of the group's and the charitable company's affairs as at 31 July 2016 and of the group's income and application of resources, including its income and expenditure, for the year then ended;
- have been properly prepared in accordance with FRS 102, The Financial Reporting Standard applicable in the UK and Republic of Ireland; and
- have been prepared in accordance with the requirements of the Companies Act 2006, the Charities and Trustee Investment (Scotland) Act 2005 and Regulations 6 and 8 of the Charities Accounts (Scotland) Regulations 2006 (amended).

Opinion on other matter prescribed by the Companies Act 2006
In our opinion based on the work undertaken in the course of our audit, the information given in the Strategic Report and the Trustees' Annual Report for the financial year for which the financial statements are prepared is consistent with the financial statements.

Matters on which we are required to report by exception

We have nothing to report in respect of the following matters where the Companies Act 2006 or the Charities Accounts (Scotland) Regulations 2006 (as amended) requires us to report to you if, in our opinion:

- the parent charitable company has not kept adequate accounting records; or
- the parent charitable company financial statements are not in agreement with the accounting records and returns; or
- certain disclosures of trustees' remuneration specified by law are not made; or
- we have not received all the information and explanations we require for our audit.

Michael Hicks
Senior Statutory Auditor
For and on behalf of
Crowe Clark Whitehill LLP
Statutory Auditor
London

Consolidated Statement of financial activities

(incorporating an income and expenditure account) For the year ended 31 July 2016

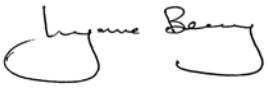
	Note	Unrestricted Funds £000	Restricted Funds £000	**Total 2016 Funds £000**	Total 2015 Funds £000
Income from:					
Donations and legacies	2	16,943	7,015	**23,958**	24,580
Income from charitable activities		604	-	**604**	-
Income from other trading activities	3	1,437	1,235	**2,672**	1,960
Income from investments		382	-	**382**	457
Total income		19,366	8,250	**27,616**	26,997
Expenditure on raising funds	4	13,172	-	**13,172**	14,127
Expenditure on charitable activities					
Risk and prevention	5	29	2,850	**2,879**	3,155
Early detection and diagnosis	5	879	371	**1,250**	3,006
Treatment	5	7,245	3,630	**10,875**	7,244
Secondary breast cancer	5	2,151	1,088	**3,239**	2,882
Total expenditure on charitable activities		10,304	7,939	**18,243**	16,287
Merger costs	18	287	-	**287**	2,935
Total expenditure		23,763	7,939	**31,702**	33,349
Net realised investment (loss)/gain		(203)	-	**(203)**	812
Net unrealised investment (loss)/gain		930	-	**930**	(41)
Total net investment (loss)/gain		727	-	**727**	771
Net income/(expenditure)		(3,670)	311	**(3,359)**	(5,581)
Transfers between funds	16	3	(3)	**-**	-
Net movement in funds		(3,667)	308	**(3,359)**	(5,581)
Funds brought forward		13,707	863	**14,570**	20,151
Funds carried forward		10,040	1,171	**11,211**	14,570

Balance sheets

as at 31 July 2016

	Note	**Group 2016 £000**	Group 2015 £000	**Charity 2016 £000**	Charity 2015 £000
Fixed assets					
Intangible assets	7	**279**	191	**279**	191
Tangible assets	8	**1,162**	1,150	**1,162**	1,150
Investments	9	**16,201**	21,005	**16,201**	21,005
Total Fixed assets		**17,642**	22,346	**17,642**	22,346
Current assets					
Current investments	10	**77**	110	**77**	110
Debtors	12	**6,952**	3,697	**7,737**	3,703
Cash at bank and in hand		**5,921**	5,870	**1,562**	5,690
Total Current Assets		**12,950**	9,677	**9,376**	9,503
Creditors: amounts falling due within one year	13	**(12,752)**	(10,811)	**(9,196)**	(10,654)
Net current assets/(liabilities)		**198**	(1,134)	**180**	(1,151)
Total assets less current liabilities		**17,840**	21,212	**17,822**	21,195
Creditors: amounts falling due after more than one year	14	**(6,629)**	(6,642)	**(6,629)**	(6,642)
Net assets		**11,211**	14,570	**11,193**	14,553
Funds					
Unrestricted	16	**10,040**	13,707	**10,022**	13,690
Restricted		**1,171**	863	**1,171**	863
Total Funds		**11,211**	14,570	**11,193**	14,553

These financial statements were approved by the Board of Trustees and authorised for issue on 15 December 2016 and were signed on its behalf by:



Professor Lynne Berry
Chair of Trustees

Consolidated cash flow statement

For the year ended 31 July 2016

	Note	£000	Group 2016 £000	£000	Group 2015 £000
Cash flows from from operating activities:					
Net cash (used in) operating activities	a		**(5,399)**		(8,920)
Cash flows from investing activities					
Dividends, interest and rents from investments		63		194	
Purchase of tangible fixed assets		(271)		(1,139)	
Purchase of intangible fixed assets		(162)		-	
Proceeds from sale of investments		5,787		3,962	
Purchase of investments		-		(7,187)	
Net cash (used in) investing activities			**5,417**		(4,170)
Change in cash in the reporting period			**18**		(13,090)
Cash at the beginning of the reporting period			**5,980**		19,070
Cash at the end of the reporting period			**5,998**		5,980
(a) reconciliation of net income to net cash flow from operating activities					
Net income / (expenditure) for the reporting period (as per the Statement of Financial Activities)			**(3,359)**		(5,581)
Depreciation and amortisation			**333**		373
Write back of provision (current asset investments)			**(22)**		-
Realised (gains)/losses on investments			**225**		-
Unrealised (gains)/losses on investments			**(930)**		(771)
Dividends, interest and rents from investments			**(382)**		(457)
Investment fees deducted from portfolio			**63**		43
Loss/(profit) on sale of fixed assets			**-**		49
Decrease/(increase) in debtors			**(3,255)**		1,625
(Decrease)/increase in creditors			**1,928**		(4,201)
Net cash (used in) operating activities			**(5,399)**		(8,920)

	Group 2016 £000	Group 2015 £000
(b) Analysis of cash and cash equivalents		
Cash in hand	**2,721**	5,870
Notice deposits (less than 3 months)	**3,200**	-
Long-term deposits	**77**	110
Total cash and cash equivalents	**5,998**	5,980

Notes to the financial statements

For the year ended 31 July 2016

1. Accounting policies

Breast Cancer Now is a Public Benefit Entity registered as a charity in England and Wales and a company which was incorporated on 09/12/2014 (company number: 1234567) and registered as a charity on 18/02/2015 (charity number: 987654). It is a registered charity in Scotland (SC045584) and Isle of Man (1200).

Basis of preparation
These financial statements are prepared under the historical cost convention, as modified by the inclusion of investments at fair value and in accordance with Financial Reporting Standard 102 ('FRS 102') - 'The Financial Reporting Standard applicable in the United Kingdom and Republic of Ireland' and with the Statement of Recommended Practice 'Accounting and Reporting by Charities' FRS 102 as revised in 2015 ('the SORP 2015'), together with the Companies Act 2006, the Charities Act 2011, the Charities and Trustee Investment (Scotland) Act 2005 and the Charities Accounts (Scotland) Regulations 2006.

Having reviewed the activities and performance of the Group the Trustees have reasonable expectation that the group has adequate resources to continue its activities for the foreseeable future. Accordingly, they have adopted the going concern basis in preparing the financial statements as outlined in the Statement of Trustees' Responsibilities.

The functional currency of the Group and charity is considered to be GBP because that is the currency of the primary economic environment in which the charity operates.

Transition to FRS 102
In preparing the accounts, the trustees have considered whether in applying the accounting policies required by FRS 102 and the Charities SORP FRS 102 the restatement of comparative items was required. In their estimation, the impact of transitioning is not material to the financial statements and therefore the restatement of comparative items is not required. The transition date was 1 August 2014.

Critical accounting judgements and key sources of estimation uncertainty
In the application of the charity's accounting policies, Trustees are required to make judgements, estimates and assumptions about the carrying values of assets and liabilities that are not readily apparent from other sources. The estimates and underlying assumptions are based on historical experience and other factors that are considered to be relevant. Actual results may differ from these estimates.

The estimates and underlying assumptions are reviewed on an on-going basis. Revisions to accounting estimates are recognised in the period in which the estimate is revised if the revision affects only that period, or in the period of the revision and future periods if the revision affects the current and future periods.

In the view of the Trustees, no assumptions concerning the future or estimation uncertainty affecting assets and liabilities at the balance sheet date are likely to result in a material adjustment to their carrying amounts in the next financial year.

Consolidation
The financial statements consolidate the charity and its trading subsidiaries. The accounts present the consolidated statement of financial activities (SOFA), the consolidated cash flow statement and the consolidated and Charity balance sheets. The accounts have been prepared on a line by line basis on the face of the statement of financial activities. The charity has taken advantage of the exemption from presenting its unconsolidated income and expenditure account under section 408 of Companies Act 2006. The net result of the charity for the year was a deficit of £3.36m (2015: deficit £5.65m)

Cash flow statement
The charity has taken advantage of the exemption available to a qualifying entity in FRS 102 from the requirement to present a charity only Cash Flow Statement with the consolidated financial statements.

Funds
The following funds are held by the Charity:

Unrestricted funds are donations and other income receivable or generated from the objects of the charity without further specified purpose and are available as general funds.

Restricted funds arise when conditions are imposed by the donor, or by the specific terms of the appeal, and can only be spent on the activities specified."

The Charity relies heavily on donations and legacies, which fluctuate year on year. In order to continue day-to-day operations the Trustees have identified that a minimum level of unrestricted funds should be maintained. This minimum level of funds is kept under annual review by the Trustees in line with the reserves policy as described in the Trustees report.

Income
Income is accounted for and included in the SOFA when the Group is entitled to the income, receipt can be quantified and receipt is probable. Income is deferred when it relates to future accounting periods.

Donations are recognised upon receipt along with any related gift aid. Legacies are taken into account when capable of financial measurement, receipt is probable and where there are no conditions that still need to be fulfilled before payment is made to us.

Income from corporate sponsorship and products is recognised when it is earned.

Income received in respect of lotteries is recognised when the draw is made. Income received in advance for future lottery draws is deferred until the draw takes place.

Gifts in kind and donated goods and services are recognised in the accounts when the benefit to Breast Cancer Now is reasonably quantifiable and measurable. The value is the price Breast Cancer Now estimates it would pay should it purchase equivalent goods or services.

Income (continued)
Investment income is recognised on a receivable basis.

The income of regional Groups includes all transactions cleared on the regional Group bank statements up to the year-end.

Income from fundraising events is recognised when the event takes place.

Expenditure
Expenditure is accounted for on an accruals basis and attributed to the appropriate activities within the statement of financial activities.

Expenditure on raising funds include direct staff costs and expenditure relating to all fundraising activities.

Expenditure on charitable activities includes direct staff costs and expenditure relating to charitable activities, as well as research grant expenditure. Research grants in furtherance of the Charity's objectives are the total amounts granted to external bodies for charitable work. The grants made by the Trustees are recognised in the statement of financial activities in the year the grant is awarded and notified to the recipient, provided a legal or constructive commitment exists and any conditions attaching to the grant have been fulfilled by the recipient.

Costs relating to the sale of goods include the direct costs of purchasing and distributing goods for sale.

Communication and support costs are reviewed and any costs directly relating to our charitable activities have been allocated to the appropriate strategic charitable priority with the remainder being allocated to charitable activities and raising funds based on a combination of headcount and staff time.

Governance costs are the costs incurred to manage the Charity in compliance with constitutional and statutory requirements and are included in support costs.

Value Added Tax
Value Added Tax is only partially recoverable by the Charity and therefore the non-recoverable element is included with the expenditure on which the VAT was charged in the statement of financial activities.

Valuation of investments
Fixed asset investments are stated at fair value at the balance sheet date. Any realised or unrealised gains and losses are shown in the statement of financial activities. Gains and losses are calculated with reference to market values as at the beginning of the year or cost if purchased during the year.

Current investments are shown at cost less any provision for expected losses.

Financial instruments
Basic financial instruments are initially recognised at transaction value and subsequently measured at amortised cost with the exception of investments which are held at fair value. Financial assets held at amortised cost comprise cash at bank and in hand, together with trade and other debtors excluding prepayments. A specific provision is made for debts for which recoverability is in doubt. Cash at bank and in hand is defined as all cash held in instant access bank accounts and used as working capital. Financial liabilities held at amortised cost comprise all creditors excluding deferred income, social security and other taxes and provisions. Assets and liabilities held in foreign currency are translated to GBP at the balance sheet date at an appropriate year end exchange rate. No discounting has been applied to these financial instruments on the basis that the periods over which amounts will be settled are such that any discounting would be immaterial.

Investments, including bonds and cash held as part of the investment portfolio are held at fair value at the Balance Sheet date, with gains and losses being recognised within income and expenditure. Investments in subsidiary undertakings are held at cost less impairment.

At the balance sheet date the charity held financial assets at amortised cost of £10,881k (2015 £8,761k). Financial assets at fair value through income or expenditure of £16,201k (2015 £21,005k) and Financial liabilities at amortised cost of £15,437k (2015 £16,734k)

Tangible fixed assets and depreciation
Tangible fixed assets are stated at cost, less depreciation. Assets of under £500 in value are not capitalised but are taken fully as expenditure in the year of purchase.

Depreciation is provided by the straight-line method, calculated to write off assets over their estimated useful lives at the following rates:

Fixtures, fittings and office equipment: over 2 to 4 years
Leasehold improvements: over the lease period
IT hardware: over 2 to 4 years

Intangible assets
Intangible assets are stated at cost less accumulated amortisation. Intangible assets of under £500 in value are not capitalised but are expensed fully in the year of purchase.

Amortisation is calculated using the straight line method at the following rate:

IT software and website: over 2 to 4 years

Operating leases
Rentals under operating leases are charged on a straight-line basis over the lease term, even if the payments are not made on such a basis. Benefits received and receivable as an incentive to sign an operating lease are similarly spread on a straight-line basis over the lease term.

Pensions
Employees are entitled to join the pension scheme provided by Aegon. This is a defined contribution scheme administered by an independent scheme administrator. Scheme funds are independent to the charity and invested with Aegon. The charity contributes by matching employee contributions to their personal pension to a maximum of 7% of salary. The cost of providing this pension scheme is charged to the statement of financial activities when it is incurred.

2. Donations and legacies

	2016 £000	2015 £000
Trusts and appeals	**1,079**	1,291
Donations and legacies from individuals	**16,525**	18,585
Corporate donations	**5,641**	3,829
Other income	**713**	875
Total	**23,958**	24,580

3. Other trading activities

	2016 £000	2015 £000
Events	**200**	3
Corporate products and sponsorship	**2,381**	1,933
Other income	**91**	24
Total	**2,672**	1,960

4. Expenditure on raising funds

	Direct costs £000	Support costs £000	**Total 2016 £000**	Total 2015 £000
Cost of raising funds from donations and legacies	10,587	1,986	**12,573**	13,255
Cost of other trading activities	453	76	**529**	805
Cost of managing investments	70	-	**70**	67
Total	11,110	2,062	**13,172**	14,127

5. Charitable expenditure

	Grants £000	Direct costs £000	Support costs £000	**Total 2016 £000**	Total 2015 £000
5a. Charitable activities					
Risk and prevention	1,879	672	328	**2,879**	3,155
Early detection and diagnosis	325	841	84	**1,250**	3,006
Treatment	8,659	1,485	731	**10,875**	7,244
Secondary breast cancer	2,284	737	218	**3,239**	2,882
Total	13,147	3,735	1,361	**18,243**	16,287

	Total 2016 £000	Total 2015 £000
5b. Grants		
Breast Cancer Now Toby Robins Research Centre (ICR)	**7,926**	4,417
Breast Cancer Now Generations Study (ICR)	**1,657**	113
Tissue Bank	**1,478**	579
Research units:		
King's College London	**738**	1,129
Manchester	**303**	568
Dundee	**149**	155
Edinburgh	**102**	89
Other Research Grants	**794**	2,864
Total	**13,147**	9,914

	2016 £000	2015 £000
5c. Support costs		
IT, Finance & HR	**1,586**	2,345
Office	**1,335**	1,658
Management	**323**	1,548
Governance	**179**	298
Total	**3,423**	5,849

	2016 £000	2015 £000
The above expenditure includes the following charges:		
Audit fee	**35**	41
Depreciation	**333**	373
Operating leases - property	**873**	864

6. Employees

	2016 £000	2015 £000
Wages and salaries	**5,929**	7,922
Social security cost	**599**	777
Pension costs	**310**	284
Total	**6,838**	8,983

Termination payments during the year totalled £91k (2015: £460k), £28k of which was outstanding at the year end. Termination payments are recognised on an accruals basis.

The average number of employees during the year were:	2016 no.	2015 no.
Charitable activities	**56**	65
Fundraising	**85**	103
Support Services and governance	**25**	39
Total	**166**	207

The number of employees receiving remuneration over £60,000 during the year was as follows:	2016 total no.	2015 total no.
£60,001 - £70,000	**2**	3
£70,001 - £80,000	**1**	2
£80,001 - £90,000	**3**	7
£90,001 - £100,000	**1**	-
£100,001 - £110,000	**-**	1
£110,001 - £120,000	**1**	-
£150,001 - £160,000	**-**	1
Total	**8**	14

Pension costs for these higher paid employees amounted to £40k (2015: £73k).

The key management personnel of the charity comprise the trustees, the Chief Executive, the Director of Finance & Corporate Services, the Director of Communications & Engagement, the Interim Director of Research, the Director of Scotland and the Director of Fundraising. The total earnings, including pension contributions, received by the key management personnel of the charity during the year were £564k (2015 - £1.02m).

7. Intangible fixed assets (Group & Charity)

	IT Software & Website £000
Cost	
At 1 August 2015	611
Additions	162
At 31 July 2016	773
Amortisation	
At 1 August 2015	420
Charge for the year	74
At 31 July 2016	494
Net book value	
Brought forward at 1 August 2015	191
Carried forward at 31 July 2016	279

8. Tangible fixed assets (Group & Charity)

	Leasehold Improvements £000	Fixtures and Fittings £000	IT Hardware £000	Total £000
Cost				
At 1 August 2015	613	167	983	1,763
Additions	244	-	27	271
At 31 July 2016	857	167	1,010	2,034
Depreciation				
At 1 August 2015	41	62	510	613
Charge for the year	46	25	188	259
At 31 July 2016	87	87	698	872
Net book value				
Brought forward at 1 August 2015	572	105	473	1,150
Carried forward at 31 July 2016	770	80	312	1,162

There were no assets held under finance leases in either year.

9. Investments (Group and Charity)

	2016 £000	2015 £000
Market value at 1 August 2015	**21,005**	16,789
Additions	**-**	7,187
Realised (losses)/gains	**(225)**	812
Unrealised (losses)/gains	**930**	(41)
Income reinvested	**341**	263
Investment fees	**(63)**	(43)
Withdrawals	**(5,787)**	(3,962)
Market value at 31 July 2016	**16,201**	21,005

Subsidiary undertaking (charity only)
The charity's investment in its subsidiaries is £144 being two ordinary shares of £1 each in BCN Trading Limited, 100 ordinary shares of £1 each in Breakthrough Enterprises Limited, one ordinary share of £1 in Breakthrough Promotions Limited, one ordinary share of £1 in BCC Trading Limited and forty shares of £1 each in Pink Ribbon Limited representing a majority shareholding. Breast Cancer Care hold the remaining twenty shares of £1 each in Pink Ribbon Limited. The other subsidiary Breast Cancer Campaign Scotland, a charitable company which has common trustees and objectives, was dormant during the year.

All investment assets are held in the UK and are unrestricted. The holdings by fund on a market value basis are as follows:

Fund	Asset class	Fund manager	2016 £000	2015 £000
Charishare	Equities	BlackRock	**3,295**	4,576
Charibond	Fixed interest	M&G	**3,207**	5,076
Coutts	Bonds, Equity & Commodities	Coutts	**4,296**	3,966
Goldman Sachs	Equity linked	Goldman Sachs	**5,403**	4,935
Barclays Wealth	Equity linked	Barclays Wealth	**-**	2,452
Total market value			**16,201**	21,005

The total amount held in listed investments as at 31 July 2016 was £10,798k.

10. Current investments (Group and Charity)

	2016 £000	2015 £000
Balance as at 1 August 2015	**110**	701
Income reinvested	**-**	7
Unrealised gains/(losses)	**-**	1
Realised gains/(losses)	**-**	-
Write back of provision	**22**	-
Transfer to cash at bank and in hand	**(55)**	(599)
Balance as at 31 July 2016	**77**	110

At 1 August 2008 the Charity held £4.37m of short-term cash deposits with Kaupthing Singer and Friedlander (KSF). A further £44k was due in interest as at that date. On 8 October 2008 the FSA applied to the High Court for KSF to be put into administration, as they concluded that it no longer met the FSA's threshold conditions. The Charity recovered £0.055m during the period, taking the total recovery to 31 July 2016 to £3.70m. This represents 83.7% of the original sum invested.

As at 7th April 2016, the administrator's estimate was updated to state that between 85.5% and 86.5% will ultimately be recovered by the charity, compared to between 85% and 86.5% in the prior year. The Trustees have therefore decided to decrease the provision for non-recovery of the original sum invested to 14.5% or £640k (2015: 15% or £662k).

11. Subsidiary undertakings

	Breakthrough Promotions Ltd £000	BCC Trading Ltd £000	BCN Trading Ltd £000	**2016 Total £000**	2015 Total £000
Turnover	-	253	2,672	**2,925**	1,960
Cost of sales	-	(253)	(48)	**(301)**	(56)
Administrative expenses	-	-	(5)	**(5)**	(154)
Profit before gift aid and taxation	-	-	2,619	**2,619**	1,750
Gift Aid payable to Breast Cancer Now	-	-	(2,619)	**(2,619)**	(1,750)
Profit before taxation	-	-	-	**-**	-
Tax expense	-	-	-	**-**	-
Profit after taxation	-	-	-	**-**	-
Assets	153	3,514	1,686	**5,353**	1,691
Liabilities	(144)	(3,514)	(1,679)	**(5,337)**	(1,675)
Net assets	9	-	7	**16**	16

Breakthrough Enterprises Ltd, Breast Cancer Campaign Scotland, Pink Ribbon Ltd, Breast Cancer Campaign and Breakthrough Breast Cancer are dormant companies. The registered name of BCC Trading Limited was changed to BCN Research Limited on 14 December 2016.

12. Debtors

	Group 2016 £000	Group 2015 £000	**Charity 2016 £000**	Charity 2015 £000
Trade debtors	**881**	1,494	**101**	210
Intercompany	**-**	-	**1,627**	1,483
Other debtors	**1,741**	892	**1,740**	892
Prepayments & accrued income	**4,330**	1,311	**4,269**	1,118
Total	**6,952**	3,697	**7,737**	3,703

13. Creditors: amounts falling due within one year

	Group 2016 £000	Group 2015 £000	**Charity 2016 £000**	Charity 2015 £000
Trade creditors	**691**	1,092	**691**	1,082
Grants payable	**7,232**	7,894	**7,232**	7,894
Taxes and social security	**468**	324	**168**	178
Accruals and deferred income	**4,307**	1,501	**1,051**	1,500
Other creditors	**54**	-	**54**	-
Total	**12,752**	10,811	**9,196**	10,654

Deferred income comprises sponsorship income received in advance of fundraising events taking place in 2016/17, as well as income received in advanced for entries to our weekly lottery and ticket sales for our Pink Ribbon Ball. Deferred income also includes income received relating to the Catalyst Programme which is run through BCC Trading Ltd, a wholly owned subsidiary of Breast Cancer Now. Income is recognised at the point research spend is committed.

In the prior year the deferred income also included ticket sales for the Bigger Bounce, which is held every other year.

	Group 2016 £000	Charity 2016 £000
Balance as at 1 August 2015	**395**	395
Amount released to income	**(395)**	(395)
Amount deferred in year	**3,411**	159
Balance as at 31 July 2016	**3,411**	159

14. Creditors: amounts falling due in more than one year

	Group 2016 £000	Group 2015 £000	**Charity 2016 £000**	Charity 2015 £000
Grants payable	**6,629**	6,642	**6,629**	6,642

15. Grants payable

	2016 £000	2015 £000
At 1 August 2015	**14,536**	18,903
Awarded during the year	**14,343**	10,930
Paid during the year	**(13,825)**	(14,281)
Adjustments during the year	**(1,193)**	(1,016)
At 31 July 2016	**13,861**	14,536
Due within one year (note 13)	**7,232**	7,894
Due after one year (note 14)	**6,629**	6,642
At 31 July 2016	**13,861**	14,536

16. Reconciliation of funds

	Balance at 1 Aug 2015 £000	Income £000	Expenditure £000	Investment Gains £000	Transfers £000	**Balance at 31 Jul 2016 £000**
Unrestricted funds	13,707	19,366	(23,763)	727	3	**10,040**
Restricted						
Tissue Bank (1)	239	1,437	(1,479)	-	-	**197**
Prevention (2)	-	3,276	(2,479)	-	-	**797**
Other restricted funds (3)	624	3,537	(3,981)	-	(3)	**177**
Total restricted funds	863	8,250	(7,939)	-	(3)	**1,171**
Total funds	14,570	27,616	(31,702)	727	-	**11,211**

Notes
(1) Donations specifically given to fund the Breast Cancer Now Tissue Bank
(2) Donations specifically given to fund the strategic theme of prevention
(3) This includes a variety of funds, where donations are restricted to specific areas of research and other charitable activity

17. Analysis of net assets between funds

	Group Unrestricted £000	Group Restricted £000	**Group 2016 £000**	Group 2015 £000
Tangible assets	1,441	-	**1,441**	1,341
Investments	16,201	-	**16,201**	21,005
Net current assets/ (liabilities)	(973)	1,171	**198**	(1,134)
Long term liabilities	(6,629)	-	**(6,629)**	(6,642)
Total net assets	10,040	1,171	**11,211**	14,570

	Charity Unrestricted £000	Charity Restricted £000	**Charity 2016 £000**	Charity 2015 £000
Tangible assets	1,441	-	**1,441**	1,341
Investments	16,201	-	**16,201**	21,005
Net current assets/ (liabilities)	(991)	1,171	**180**	(1,134)
Long term liabilities	(6,629)	-	**(6,629)**	(6,642)
Total net assets	10,022	1,171	**11,193**	14,570

18. One-off merger costs

	2016 £000	2015 £000
Communications and marketing	**172**	1,453
Redundancy related costs	**24**	550
Legal and professional	**-**	416
Property costs and asset write off	**30**	395
System integration	**61**	121
Total net assets	**287**	2,935

19. Financial and other commitments

Operating lease commitments
The Group and Charity have the following future minimum lease payments under non-cancellable operating leases for each of the following periods:

	2016 £000	2015 £000
Property leases expiring:		
Within one year	**716**	749
Within two to five years	**1,722**	2,438
In over five years	**-**	-
Equipment leases expiring:		
Within one year	**9**	45
Within two to five years	**15**	-
In over five years	**-**	-

20. Trustee expenses

The trustees received no remuneration in the year. 3 (2015:3) Trustees received reimbursement for travel and accommodation expenses totalling £1,646 for the year (2015: £834).

21. Related party disclosures

In accordance with FRS 102, the charity discloses related party transactions that were recognised in the statement of financial activities.

During the year Breast Cancer Now Trustee Professor Adrian Llewellyn Harris was a Non-Executive Director of the Institute of Cancer Research (ICR). Total grants of £9.4m were awarded to The Institute in the year ending 31 July 2016 (2015 £4.4m). During the year the charity paid for services totalling £46,183 (2015: £24,950) to Liz Hobbs Group Limited, a company in which Liz Doogan-Hobbs is a Director and has a controlling interest. The services were provided at arm's length and were for the provision of production and event management services.

During the year, the charity had the following transactions with its subsidiary companies:

- Payment under gift aid from BCN Trading of £2,619k.
- Management charge receivable from BCC Trading of £253k.

At the year end, the charity had an outstanding debtor balance due from BCN Trading of £1,374k and an outstanding debtor balance due from BCC Trading of £253k.

22. Comparative Statement of Financial Activities

	Unrestricted funds £000	Restricted funds £000	**Total 2015 £000**
Income from:			
Donations and legacies	19,582	4,998	**24,580**
Income from other trading activities	1,960	-	**1,960**
Income from investments	457	-	**457**
Total income	21,999	4,998	**26,997**
Expenditure on raising funds	14,127	-	**14,127**
Expenditure on charitable activities			
Risk and prevention	2,146	1,009	**3,155**
Early detection and diagnosis	2,459	547	**3,006**
Treatment	4,152	3,092	**7,244**
Secondary breast cancer	1,422	1,460	**2,882**
Total expenditure on charitable activities	10,179	6,108	**16,287**
Merger costs	2,935	-	**2,935**
Total expenditure	27,241	6,108	**33,349**
Net realised investment (loss)/gain	(41)	-	**(41)**
Net unrealised investment (loss)/gain	812	-	**812**
Total net investment (loss)/gain	771	-	**771**
Net income/(expenditure)	(4,471)	(1,110)	**(5,581)**
Transfers between funds	918	(918)	**-**
Net movement in funds	(3,553)	(2,028)	**(5,581)**
Funds brought forward	17,260	2,891	**20,151**
Funds carried forward	13,707	863	**14,570**

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Director: Professor Andrew Tutt

Thank you

We rely upon the support we receive from organisations and individuals who so generously give their time and money to support our work, and we would like to pay special thanks to the following individuals and organisations:

We thank all breast cancer patients who have kindly donated tissue and blood samples to our Tissue Bank. These samples are invaluable to the scientific community and will help drive new discoveries in breast cancer research.

We thank the staff and scientists in the laboratories we fund who have taken time to show people around their labs and talk about the work that Breast Cancer Now is funding.

We thank the clinicians, scientists and supporters who helped us develop our health information.

We thank our Scientific Advisory Board (SAB) and expert peer reviewers from all over the globe who helped us evaluate the grant applications. Both the SAB and peer reviewers donate their time for this important task.

We thank Professor Robert Coleman, Professor of Medical Oncology, University of Sheffield and Dr Catherine Harper-Wynne, Secretary of UK Breast Cancer Group and Consultant Medical Oncologist, Kent Oncology Centre for their help and support on our bisphosphonates campaign.

We thank Mr Nick Thomas-Symonds MP for his support of the Off-Patent Drugs Bill in parliament and continuing to raise issues relating to off-patent drugs. Alun Cairns MP who ran the marathon for us.

We thank Connie Johncock and Mary Huckle for their support on our Access to Medicines campaigns and Jayne Hawkins, our patient expert for the NICE appraisal of Perjeta neoadjuvant.

We thank The All-Party Parliamentary Group on Breast Cancer officers: Mrs Sharon Hodgson, Craig Tracey, Dr Philippa Whitford, Jo Churchill, Maria Caulfield.

We thank sll leaders of Scotland's five main political parties for their collective commitment to stopping deaths from breast cancer by 2050.

We thank all MSPs who have shown their support for our campaigns over the last year.

We thank Scotland's Chief Scientist's Office

We thank all of our supporters who had the courage to tell their stories to the media as part of our campaigns and influence decision makers to help future generations.

We also acknowledge the generous support of:

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Walk the Walk
Khoo Teck Puat UK Foundation
Mary-Jean Mitchell Green Foundation
Garfield Weston Foundation
The Haley Family Charitable Trust
Department of Health
The Pink Ribbon Ball Committee London
The Pink Ribbon Ball Committee Manchester
A Bigger Bounce Ball Committee
Loughborough Rag
Nottingham Karnival
Kent RAG
Art for Cure
Mark Urquhart
Sally Burton

Every individual, local group, RAG, charitable trust and company who has donated or raised money for us in the past year, or remembered us in their Will. It is only with their support that we are able to fund the pioneering research which will save the lives of women with breast cancer.

We are family...

We're determined to stop women dying from breast cancer.

We know it can be done. But it's only by standing together – as scientists, policy makes, fundraisers, families, colleagues and campaigners – that we will make it happen.

breast cancer now

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